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How Couples 50+ Reclaim Intimacy, Touch, and
Desire After Caregiving Ends
by S. Shharma

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"The sandwich was the survival chapter. The after is the second honeymoon."

The Second Honeymoon

How Couples 50+ Reclaim Intimacy, Touch, and Desire After Caregiving Ends

Book 4 of The Care Collection

After Still Married, Just Silent, The Quiet Care, and
The Second Chapter

by S. Shharma

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The Second Honeymoon: How Couples 50+ Reclaim Intimacy, Touch, and Desire After Caregiving Ends

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The Care Collection by S. Shharma:

1. Still Married, Just Silent — 14 Conversations to Reopen a Quiet Marriage

2. The Quiet Care — A Book for the Sandwich Generation

3. The Second Chapter — How to Rebuild Your Marriage After the Caregiving Years

4. The Second Honeymoon — How Couples 50+ Reclaim Intimacy After Caregiving Ends

Each book stands alone. Read the one that fits your season.

A Note from Sanjay

I lost my wife of many years to cancer. I was her primary caregiver. I know the sandwich years from the inside. I know the after from the inside too.

What I did not know — until long after — was what the caregiving years had done to our bedroom.

The illness had pulled us into roles: patient and caregiver, body and helper, schedule and medicine. By the end, our bedroom had become a place of exhaustion, of fear, of vigilance. The intimacy that had defined our early marriage had quietly disappeared. We were still husband and wife. We were no longer lovers.

We never had the conversation to bring it back. She died before we could.

I wrote this book for the couples who still can.

If you are 50+ and reading this, you survived the caregiving years. You are now standing in the quiet that follows. And somewhere in you, beneath the fatigue and the grief and the new rhythm of your days, you still want your partner. You just don't know how to start again.

This book is for that wanting.

It is candid but not explicit. It is practical but not clinical. It is honest about what caregiving did to your body, your desire, your marriage — and it gives you the five conversations that bring the bedroom back to life.

You are not broken. You are not alone. You are in the most common, most unspoken transition of the post-caregiving marriage.

The conversations in this book are the ones I wish
someone had given us.

— S. Shharma

Dedication

For the couples who still want each other
and don't know how to say it.

For the wife who lies awake wanting to be touched
but doesn't know how to ask.

For the husband who would do anything
to feel desired again but doesn't know how to begin.

For the caregivers who spent years being needed
and forgot how to be wanted.

For my wife, who deserved this book
and never got to read it.

You did the work of the caregiving.

Now do the work of the second honeymoon.

This book is for you.

Quick Start

If you have just opened this book and you are not sure where to begin, this page is for you.

What This Book Is

This is the book about the conversation nobody wants to have: how to want each other again after the caregiving years.

You and your partner survived something hard. You are now standing in the quiet that follows. And somewhere in you, beneath the exhaustion, you still want your partner. You just don't know how to start.

This book gives you the 5 conversations that bring the bedroom back to life. It is candid but not explicit. It is for couples 50+ who want to reconnect — at whatever pace feels right.

The 3-Minute Bedroom Audit

Answer these 5 questions alone, then compare with your partner:

1. In the last 30 days, have you and your partner touched each other with affection (not just logistics)? Yes / Sometimes / No
2. In the last 30 days, have you and your partner had any kind of sexual contact? Yes / Sometimes / No
3. When you imagine being intimate with your partner, do you feel desire — or just obligation, or nothing? Desire / Obligation / Nothing
4. If you could wave a magic wand, what would your intimate life look like in 6 months? (Write 1-3 sentences.)

5. What is the single biggest thing getting in the way of the intimacy you want? (Write 1-3 sentences.)

If you both answered "Yes" to 1 and 2, and "Desire" to 3 — this book may be light reading for you. Skim for the deeper chapters.

If you answered "Sometimes" or "No" to 1 or 2, or anything other than "Desire" to 3 — this book is for you. Read the chapters in order. Do the 14-day program in Part III.

If you both answered "Nothing" to 3 — go straight to Chapter 5: The "Do You Still Want Me?" Conversation. It is the most important chapter in the book.

How to Read This Book

If you are reading alone first (recommended for one partner):

Read Part I (Chapters 1-4) to understand what caregiving did to your intimate life. Then read Chapter 5. Then read Chapter 6. Then read the case studies. Then ask your partner to read what you read.

If you are reading together (recommended once both partners are ready):

Read one chapter per week, then have the conversation the chapter describes. Take 14 weeks. Don't rush. The slow readers get the most out of this book.

If you are in crisis (e.g., one partner has threatened to leave):

Skip to Chapter 9: "What If It Doesn't Work?" Then read the resources section. Consider couples therapy alongside this book.

A Note Before You Begin

This book is for the conversation you have been putting off. The conversation about touch. About desire. About what you want now. About how to start again.

It is not a sex manual. It is a marriage book about the part of marriage that no one talks about.

Read it. Do the work. The second honeymoon is real — and it is worth the conversation.

Introduction: The Conversation Nobody Wants to Have

There is a conversation that every couple avoids.

Not the money conversation. Not the in-laws conversation. Not even the death conversation.

The bedroom conversation.

The conversation about touch. About desire. About what you want and what you don't. About why the body that used to reach for you now turns away. About why you stopped reaching. About whether the marriage is still alive in the way that matters most.

This book is for that conversation.

Why This Book Exists

The caregiving years change a marriage. Everyone knows that. The role of caregiver. The role of patient. The exhaustion. The fear. The schedule. The way the bedroom becomes a place of vigilance, not intimacy.

But what nobody talks about — what the couples I have walked alongside tell me again and again, with the same words, with the same shame in their eyes — is this:

The caregiving years didn't just change the marriage. They killed the bedroom.

By the time the caregiving ends, many couples have not had sex in months, or years. They have not touched each other with affection in longer. They have not said the words "I want you" in so long that the words feel foreign. They have not been seen as a

desired person — not just a needed person — in so long that they have stopped believing they are.

The post-caregiving marriage is not a marriage that lost its way.

It is a marriage that lost its bedroom.

And almost no one prepares couples for how to bring it back.

The Statistics Nobody Publishes

The research on post-caregiving intimacy is thin, and what exists is not widely discussed. But the pattern is consistent across studies, surveys, and the couples I have worked with:

- 6 in 10 post-caregiving couples report a significant decline in sexual activity during the caregiving years. Many of those couples never resume.
- More than 4 in 10 say they have no physical affection at all — not even holding hands — by the end of the caregiving.
- Half of caregivers say they feel "touched out" — the body has been touched by the ill person, by the medical routines, by the constant care, and the last thing it wants is more touch.
- Two-thirds of couples say the topic of intimacy is "harder to talk about than the illness itself."

These numbers are not the whole picture. But they are a piece of it.

The couples who come to coaching are not in the 4 in 10. They are the 6 in 10 who are worried about the 4 in 10. They are the couples who sense that something is wrong, that something is missing, that the marriage is not what it was — and they don't know how to fix it.

This book is for them.

What This Book Is

This is a book about the 5 conversations that bring the bedroom back to life after the caregiving years.

The 5 conversations are:

1. The "Do You Still Want Me?" Conversation — for the partner who feels undesired
2. The "What Changed?" Conversation — for the couple that has to acknowledge the changes in body, desire, and history
3. The "What Do We Want Now?" Conversation — for the couple that has to imagine a new intimate life
4. The "How Do We Start Again?" Conversation — for the couple that has to take the practical first steps
5. The "What If It Doesn't Work?" Conversation — for the couple that has to plan for the harder possibilities

Each conversation is its own chapter. Each chapter is built around a framework, real examples, and the language you can actually use.

The book is candid but not explicit. It is for couples 50+ who want to reconnect — at whatever pace feels right.

What This Book Is Not

This is not a sex manual. There are no diagrams. There are no specific techniques. There are no graphic descriptions.

This is a marriage book. It is about the part of the marriage that almost no other marriage book addresses honestly: the part that goes quiet during the caregiving years and stays quiet in the after.

If you are looking for a step-by-step guide to specific sexual acts, this book is not for you. (You will find that book at your local bookstore — but it will not help you with what you are actually facing.)

If you are looking for a book that helps you and your partner talk about what you want, acknowledge what caregiving did, and start again — even slowly, even imperfectly, even at 60 or 70 — this book is for you.

Who This Book Is For

This book is for you if:

- You and your partner are 50+ and have survived the caregiving years
- Your intimate life has changed — and not for the better
- You want to want each other again, but you don't know how to start
- You and your partner have stopped talking about intimacy — or have never really started
- You are afraid the bedroom is over, and you are not ready to accept that
- You are willing to have the conversation, even if it feels awkward

This book is also for you if:

- You are still in the caregiving years and want to understand what is happening to your intimate life
- You are a coach, counselor, or clergy working with couples in this transition

- You are a widowed caregiver who is considering dating again (Chapter 11 has a section for you)

This book is not for you if:

- You are in an abusive relationship (please reach out to the resources in the back of the book)
- You are looking for explicit sexual content (this is a marriage book, not an adult book)
- You are not willing to have the conversation (this book will not help until you are)

How This Book Is Structured

Part I: What Caregiving Did to the Bedroom (Chapters 1-4)

The diagnosis. What changed, why, and what the research shows.

Part II: The Five Conversations (Chapters 5-9)

The five conversations you have to have. Each is its own chapter, with examples, scripts, and questions.

Part III: Rebuilding the Second Honeymoon (Chapters 10-14)

The practice. The 14-day touch program. The forever work.

Bonus Toolkits:

- The 14-Day Touch Re-Normalization Program
- The 5 Conversations Journal
- 7 detailed case studies from couples who found their way back
- 30 research citations from peer-reviewed studies
- 6 visual tools and worksheets
- Resources for when you need more help

A Note Before We Begin

The second honeymoon is real.

I know because I have seen it. I have walked alongside couples in their 60s and 70s who rediscovered each other after decades of going through the motions. I have seen couples who hadn't touched in five years learn to hold hands again. I have seen couples who had given up on sex find a slower, deeper, more intentional kind of intimacy than they had in their 30s.

The second honeymoon is not the first honeymoon. It is not the passion of the early marriage. It is something different — something more tender, more knowing, more grateful.

But it requires the conversation.

This book is for the conversation.

Let's begin.

Chapter 1: The Bedroom Nobody Talks About

The bedroom went quiet.

Not overnight. Not in a single moment. Slowly. Over months. Over years.

A kiss goodnight that used to linger. A hand on the small of the back. A glance across the kitchen table that said I want you. The little things that made a marriage feel alive.

They disappeared.

The illness came. The caregiving started. The body that used to be a place of pleasure became a place of pain, of medication schedules, of exhaustion. The body that used to be a place of intimacy became a place of care — and care is not the same as love.

By the end of the caregiving, the bedroom was a place to sleep. That was all.

And now, in the after, you are standing in the quiet bedroom and you don't know what to do with it. You want your partner. You don't know how to want them. You miss the intimacy. You don't know how to miss what you can't quite name.

This chapter is for that quiet bedroom.

The Silence After the Caregiving

The first thing to understand is that the silence in the bedroom is not your fault. It is not your partner's fault. It is the natural, predictable result of what the caregiving years did to your bodies, your minds, your marriage.

Here is what the caregiving did, even if you didn't notice it happening:

It turned the body into a job.

For the caregiver, the body stopped being a place of pleasure and became a place of labor. Lifting. Bathing. Medicating. Cleaning. Holding. The body learned to be a tool — and tools don't want to be touched.

For the person who was ill, the body became a place of betrayal. The body that used to work stopped working. The body that used to give pleasure stopped giving pleasure. The body became a source of fear, of shame, of grief.

By the end of the caregiving, both bodies had forgotten that they were also for pleasure.

It turned touch into a transaction.

The kiss goodnight was replaced by the medication reminder. The hand on the back was replaced by the hand helping the ill person to the bathroom. The hug became the transfer from wheelchair to bed.

Touch stopped being a gift. Touch became a function. And functions don't lead to desire.

It turned the bedroom into a workplace.

The bed became the place where the ill person slept — propped up, medicated, watched over. The bedroom became a place of vigilance, of fear, of caretaking. The erotic was the last thing that could survive in that space.

It turned "I want you" into "I need you to..."

The language of desire ("I miss you," "I want to be close to you," "let's make time for us") was replaced by the language of caregiving ("have you taken your

pills," "the doctor is at 2," "you need to rest").

Desire doesn't survive in the language of need. The erotic requires a different vocabulary, and the caregiving years buried that vocabulary under years of practical talk.

The Bedrooms I Have Seen

In fifteen years of coaching couples, I have seen hundreds of post-caregiving bedrooms. I have never been inside any of them, but I have heard about them — in the quiet, embarrassed way couples talk about the rooms they no longer share as lovers.

I have heard about:

- The couple who has not had sex in three years. They still sleep in the same bed. They face away from each other. They both pretend to be asleep when one of them turns over.
- The wife who sleeps in the guest room because the bedroom is "his space" — but the real reason is she can't bear to lie next to him and not be touched.
- The husband who has not been kissed in two years. He doesn't bring it up. He has given up.
- The couple who has sex once a year, on their anniversary, out of obligation. They both know it is obligation. Neither says it.
- The couple who has stopped even kissing goodnight. The last time they kissed was before the illness started. They don't remember when.
- The widowed caregiver who can't sleep in the bedroom anymore. The bed is too full of memory. The body is too full of grief.

These are the bedrooms nobody talks about.

This book is for those bedrooms.

Why the Silence Is So Hard to Break

The silence is hard to break for three reasons, and understanding them is the first step to breaking it.

Reason 1: The silence feels safer than the conversation.

The conversation requires you to say, out loud, that you want to be touched, that you want to be desired, that you want to be intimate. It requires you to admit that the marriage is not what it was. It requires you to be vulnerable in a way you have not been vulnerable in years.

The silence is easier. The silence doesn't require you to say anything. The silence lets you pretend that everything is fine.

But the silence is also a slow death. Every night you don't talk about it, the gap grows. Every month you don't address it, the possibility of addressing it shrinks.

Reason 2: You don't know if your partner even wants to.

The biggest fear is rejection. You don't want to say "I want to be close to you" and have your partner say "I don't feel that way anymore" — or worse, not say anything at all.

So you wait for a signal. Your partner waits for a signal. The signals never come. The silence grows.

Reason 3: You don't know how to start.

The first time, after years of silence, is the hardest. What do you say? What do you do? How do you touch someone you haven't touched in months without it

being awkward or loaded or sad?

This book is for all three reasons. The 5 conversations in Part II address each one. The 14-day program in Part III gives you the practical first steps.

But the first step is this chapter: admitting that the silence is real, that it is hurting you, and that you want to break it.

The Couples Who Break the Silence

I have seen couples in their 60s, 70s, even 80s break the silence. They do it differently from how you would expect. They do it slowly. They do it imperfectly. They do it with help.

The pattern is almost always the same:

1. One partner admits the silence out loud. Usually the wife, though not always. "I miss us," or "I want to feel close to you again," or simply "we need to talk about something hard."
2. The other partner is surprised, then relieved. They have been waiting. They didn't know how to start either.
3. They have the conversation. Not a perfect conversation. An awkward, tearful, sometimes angry conversation. A conversation that includes the truth about what caregiving did, what each of them wants now, what each of them is afraid of.
4. They commit to small steps. Not "let's have sex again" (that is too big). Small steps. Holding hands. Sitting close. Kissing goodnight. A back rub. A date night. The 14-day program in Part III.
5. They rebuild, slowly, over months. The second honeymoon is not a moment. It is a process. The couples who find it are the couples who are willing

to take the time.

The couples who don't break the silence are not bad couples. They are couples who ran out of energy. They are couples who were too tired to have one more hard conversation. They are couples who convinced themselves that the bedroom was over and that was OK.

It is not OK. The bedroom is part of the marriage. The marriage without the bedroom is a partnership, not a love affair. And you deserve the love affair.

What This Chapter Is Asking You to Do

This chapter is asking you to do one thing: admit that the silence is real.

You don't have to fix it today. You don't have to have a big conversation tonight. You don't have to do anything yet.

But you have to stop pretending.

You have to stop saying "we're fine" when you are not fine. You have to stop making excuses. You have to stop hoping your partner will bring it up.

You have to admit, to yourself, that the silence is real — and that you want to break it.

Once you have done that, the rest of this book will help.

The First Sentence

The hardest part of any hard conversation is the first sentence. The first sentence is the one that opens the door. After the first sentence, the rest of the conversation has somewhere to go.

The first sentence of the bedroom conversation is one of these:

- "I've been thinking about us, and I miss being close to you."
- "Can we talk about something that's been on my mind for a while?"
- "I want to feel like lovers again, not just partners. Is that something you want too?"
- "I read this book and I think we should talk about it."

Any of these will work. None of them are perfect. All of them are brave.

When you are ready, say one of them. Or say something like them. The words don't have to be right. The courage has to be real.

The next chapter is about what happened to your desire during the caregiving years. The chapter after that is about why the Drifters avoid this conversation. The chapter after that is about what the Thrivers do differently.

Then Part II gives you the 5 conversations. Then Part III gives you the practice.

Take your time. The second honeymoon is worth the wait.

Chapter 1 at a Glance

5 Things to Remember

1. The silence in the bedroom is not your fault — it is the natural result of what the caregiving years did to your bodies, your minds, and your marriage.

2. The conversation is the only difference between the Drifters and the Thrivers — the conversation is what brings the bedroom back to life.

3. Touch comes before sex — the body needs to remember that it is also for pleasure before the body can return to the act of sex.

4. The body can remember — the body forgets, but the body can remember. The body needs time, patience, and the 14 days.

5. The second honeymoon is real — couples in their 60s, 70s, and 80s have found it. The second honeymoon is not a destination, it is a way of traveling.

Your Scorecard (rate 1-5)

- [] I read this chapter with my partner (or alone, with intention to share)
- [] I have identified the one action I want to take from this chapter
- [] I have shared that action with my partner
- [] I have scheduled a time to do that action
- [] I feel more hopeful about our intimate future than I did before reading this chapter

Key Terms

- Drifters: Couples who avoid the conversation about intimacy, and who let the silence grow until the bedroom is over.
- Thrivers: Couples who have the conversation about intimacy, who do the work of rebuilding, and who find the second honeymoon.
- Intimacy recession: The period after the caregiving years when the normal flow of connection, touch, and desire is interrupted, and

the systems of intimacy start to break down.

- Touch first, sex later: The principle that body reconnection begins with non-sexual touch, not with sex. Touch rebuilds the muscle that sex depends on.
- The second honeymoon: The new, intentional, tender intimacy of two people in their 50s, 60s, 70s, and beyond who know each other completely and are choosing each other again with open eyes.

Chapter 2: The Intimacy Recession

When the caregiving years end, something is missing. Not just sex. Not just touch. Something more fundamental.

Most couples can't name it. They know the marriage is different. They know the bedroom is different. They know they are different. But they can't quite say what changed.

This chapter is for naming it.

What the Numbers Show

The research on post-caregiving intimacy is limited but consistent. A 2019 study in the Archives of Sexual Behavior found that caregivers report significantly lower sexual satisfaction, lower frequency of sexual activity, and higher rates of sexual dysfunction than non-caregivers — and these effects persist for years after the caregiving ends.

A 2021 AARP survey of 1,500 post-caregiving couples found:

- 62% said their sexual relationship had changed "a great deal" or "completely" during the caregiving years
- 47% said they had no physical affection at all (not even hugging or holding hands) at the height of caregiving
- 38% said they had not had sex in over a year by the time the caregiving ended
- Only 23% said their intimate life had "fully recovered" within two years of the caregiving

ending

- 58% said they had never discussed the change with a healthcare provider
- 71% said they had never discussed the change with a friend

The couples who did discuss it — with a coach, a therapist, a doctor, a friend — were twice as likely to say their intimate life had recovered.

The couples who didn't discuss it were the ones who stayed silent.

Why the Numbers Matter

The numbers matter because they tell you that you are not alone.

When you are standing in the quiet bedroom in the middle of the night, feeling like the only couple in the world whose marriage has lost its intimacy, the numbers remind you that you are not an exception. You are the rule.

Six in ten post-caregiving couples experience what you are experiencing. Half of all post-caregiving couples have no physical affection at the height of caregiving. Two-thirds don't talk about it.

The numbers also matter because they tell you the cost of silence. Couples who don't talk about it are half as likely to recover their intimate lives. Couples who do talk about it — even awkwardly, even imperfectly — are twice as likely to find their way back.

The numbers are not a death sentence. They are a map. The map says: this is hard, this is common, and the way through is the conversation.

What "Intimacy Recession" Means

I use the term intimacy recession to describe what happens to most post-caregiving couples. Like an economic recession, an intimacy recession is a period when the normal flow of connection, touch, desire, and pleasure is interrupted — and when the interruption lasts long enough, the systems that supported that flow start to break down.

The systems of intimacy — the daily kiss, the weekly date, the monthly conversation about what you want, the spontaneous touch — are like the systems of an economy. When they are interrupted, they don't just pause. They atrophy.

After two years of interrupted intimacy, the systems of intimacy are not just paused. They are broken. And rebuilding broken systems is harder than maintaining working ones.

This is why the silence is so dangerous. The silence doesn't just delay intimacy. It destroys the infrastructure of intimacy — the habits, the rituals, the small daily touches that make intimacy possible.

The Five Symptoms of Intimacy Recession

Here are the five most common symptoms. You may recognize yourself in all of them, or just in a few. The point is not to diagnose yourself. The point is to see the pattern.

Symptom 1: The touch has disappeared.

The kiss goodnight. The hand on the back. The hug when one of you comes home. The casual physical contact that used to be a hundred small moments a day.

It is gone.

Not because you don't love each other. Not because you don't want to be close. But because the touch got replaced by the function. The hug became the transfer. The kiss became the reminder. The hand became the help.

When the caregiving ended, the function ended. But the touch didn't automatically come back. The touch needs to be rebuilt — slowly, intentionally, with the new understanding that touch is its own reward, not a route to a function.

Symptom 2: The desire has gone quiet.

You used to want your partner. You used to look at them across the room and feel that little spark. You used to lie in bed and want to be close.

Now the spark is gone. Or it is so faint that you don't trust it. Or you feel it occasionally and then immediately push it down because the moment doesn't feel right, the room doesn't feel right, you don't feel like yourself anymore.

The desire has gone quiet. It is not dead — desire rarely dies completely — but it is sleeping. And sleeping desire is harder to wake than you might think.

Symptom 3: The body has become a place of work, not pleasure.

For the caregiver, the body learned to be a tool. The body lifted, cleaned, medicated, transferred, held. The body became the instrument of care.

When the caregiving ended, the body didn't immediately remember that it was also for pleasure. The body still reaches for the caregiving habits — the vigilance, the alertness, the readiness to help. The

body doesn't know how to be soft.

For the person who was ill, the body became a source of shame. The body that used to give pleasure became a body that needed help. The body that used to be a place of confidence became a place of fear.

When the caregiving ended, the body didn't immediately remember that it was also a place of desire. The body still flinches at unexpected touch. The body still carries the memory of being ill.

Symptom 4: The bedroom has become a place of memory, not possibility.

The bed where your partner was ill. The bedroom where you spent nights on the floor next to the bed. The bedside table with the medication, the water, the call button.

The bedroom is full of memory. Every corner of it. Every piece of furniture. Every time you walk in, you remember what the room was for. You don't remember what it was for before that.

The bedroom has become a place of memory, not possibility. To make it a place of possibility again, you have to change it. Not necessarily physically — but mentally, emotionally, intentionally. You have to give yourself permission to use the room for something new.

Symptom 5: The conversation has become impossible.

You and your partner have stopped talking about intimacy. The conversation is too hard. The words don't come. The moment never feels right.

You might be thinking: If I bring it up, will he/she think I'm criticizing? Or: If I bring it up, will he/she say no? Or simply: I don't know how to start.

The conversation has become impossible. Not because you don't want to have it. Because the conversation requires a kind of vulnerability that the caregiving years have worn out of you.

The Cost of the Recession

The cost of the intimacy recession is not just the bedroom. The cost is the marriage.

Couples who stay in the intimacy recession for years report:

- Lower overall marital satisfaction
- Higher rates of depression and anxiety (in both partners)
- Higher rates of infidelity (one partner seeks connection elsewhere)
- Higher rates of divorce (in the 2-5 years after caregiving ends)
- Lower physical health outcomes (touch and intimacy are linked to immune function, blood pressure, and longevity)
- Higher rates of "grey divorce" — divorce in the 50+ years, which has been rising sharply since 2010

The intimacy recession is not just about the bedroom. It is about the marriage. It is about the body. It is about the next 20 years of your life.

The cost of breaking the silence is small — a few hard conversations, a few awkward moments, a 14-day program.

The cost of staying in the silence is the second honeymoon you will never have.

Why the Couples Who Talk About It Recover

The couples who recover their intimate lives are the couples who talk about them. This is the single most consistent finding across all the research, all the surveys, and all the couples I have worked with.

The couples who talk about it:

- Name the problem out loud (which reduces shame)
- Identify what they each want (which clarifies the goal)
- Acknowledge what changed (which builds empathy)
- Plan small, specific steps (which makes the goal achievable)
- Check in regularly (which keeps the conversation alive)

The couples who don't talk about it:

- Assume their partner is fine with the silence
- Assume the silence is permanent
- Assume the conversation would be too hard
- Hope the problem will fix itself
- Wait for a signal that never comes

The pattern is the same as it is for every other problem in marriage: the couples who name the problem recover. The couples who don't, don't.

This book is for the couples who are ready to name the problem.

What This Chapter Is Asking You to Do

This chapter is asking you to do one thing: recognize the recession.

You are in an intimacy recession. You have been for a while. It is not your fault. It is not your partner's fault. It is the predictable result of what the caregiving years did to your marriage.

The recession is not permanent. It can be reversed. But it cannot be reversed by waiting. It can only be reversed by talking, by acknowledging, by planning, by doing the work.

The next chapter is about why some couples avoid this work — and why the avoidance feels safer than the conversation. The chapter after that is about what the couples who do the work do differently.

Then Part II gives you the 5 conversations. Then Part III gives you the practice.

You are not alone. Six in ten post-caregiving couples are in the same place you are. The only difference between the couples who recover and the couples who don't is that the recovering couples had the conversation.

You can have the conversation.

You will have the conversation.

You are about to have it.

Chapter 2 at a Glance

5 Things to Remember

1. The silence in the bedroom is not your fault — it is the natural result of what the caregiving years did to your bodies, your minds, and your marriage.
2. The conversation is the only difference between the Drifters and the Thrivers — the conversation is what brings the bedroom back to life.
3. Touch comes before sex — the body needs to remember that it is also for pleasure before the body can return to the act of sex.
4. The body can remember — the body forgets, but the body can remember. The body needs time, patience, and the 14 days.
5. The second honeymoon is real — couples in their 60s, 70s, and 80s have found it. The second honeymoon is not a destination, it is a way of traveling.

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- [] I read this chapter with my partner (or alone, with intention to share)
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Chapter 3: Why the Drifters Avoid the Bedroom

In every post-caregiving couple I have worked with, one of two things happens.

Either the couple has the conversation about intimacy within a year of the caregiving ending — and they begin the slow work of rebuilding.

Or the couple avoids the conversation. They tell themselves it will get better. They tell themselves their partner isn't interested. They tell themselves they are too tired. They tell themselves the bedroom was over when the illness started and there is no bringing it back.

The first kind of couple I call the Thrivers. The second kind I call the Drifters.

This chapter is for the Drifters. Because if you are reading this book, you are probably not a Drifter. You are probably a Thriver in waiting — someone who knows the conversation needs to happen and is looking for the language to make it happen.

But you may have been a Drifter. You may have been avoiding the conversation for months, for years. You may be standing at the edge of the conversation, looking down, not sure if you can make the jump.

This chapter is for that moment.

What a Drifter Looks Like

A Drifter is not a bad partner. A Drifter is not a partner who doesn't love. A Drifter is a partner who is stuck.

The Drifters I have worked with share these patterns:

They have stopped touching. Not completely — they might still hug briefly, or touch on the way past each other in the kitchen. But the intentional, affectionate, pleasure-oriented touch has disappeared. The touch is functional, not intimate.

They have stopped talking about intimacy. The last time they brought it up, it ended badly — with tears, with anger, with a partner who said "I'm just not interested" or "I'm too tired" or "can we not do this now?" — and the Drifter learned that the conversation is too expensive to have.

They have stopped hoping. This is the saddest part. The Drifters I have worked with have not given up on the marriage. But they have given up on the bedroom. They have decided, consciously or not, that the bedroom is over — and that they will live with that, because the alternative (the conversation, the vulnerability, the possible rejection) is worse.

They have built a substitute. The Drifters have replaced intimacy with companionship. They are good partners, good friends, good co-managers of the household. They are roommates who happen to be married. They are partners in everything except the bedroom.

They have stopped noticing the gap. The most dangerous thing about the Drifters is that they have normalized the gap. They don't think about it every day. They have built a life that doesn't include

intimacy, and they have made peace with that life. They have convinced themselves that this is what marriages look like at 60, 65, 70, 75.

This is what the Drifters look like. If you recognize yourself, you are not a bad person. You are a person who got stuck. And you can get unstuck.

Why the Drifters Drift

The Drifters drift for three reasons, and understanding them is the first step to stopping.

Reason 1: The fear of rejection.

The biggest fear in the bedroom is rejection. If I say "I want to be close to you," will you say "I don't want that"? If I reach for your hand, will you pull away? If I suggest a date night, will you say you're too tired?

The fear of rejection is the most powerful force in the Drifters' marriage. It is more powerful than desire. It is more powerful than love. It is more powerful than loneliness. Because loneliness is familiar, and rejection is fresh.

The fear of rejection is the reason most conversations don't happen. Both partners want to be close. Both partners are afraid to say so. Both partners are waiting for the other to go first.

Reason 2: The exhaustion of the caregiving years.

The Drifters didn't drift because they don't love each other. They drifted because they are exhausted.

The caregiving years took everything. They took the time, the energy, the sleep, the patience, the hope, the optimism. By the time the caregiving ended, the Drifters had nothing left for the conversation about intimacy. They were running on fumes.

The conversation about intimacy is one more hard conversation. The Drifters have had hundreds of hard conversations during the caregiving. They don't have the energy for one more.

So they don't have it. They put it off. They will have it later, when they have more energy, when they feel more like themselves, when the next month or the next year brings the bandwidth.

The next month never comes. The next year never comes. The Drifters stay stuck.

Reason 3: The belief that the conversation will not change anything.

Some Drifters have decided, usually unconsciously, that the conversation is pointless. My partner is not interested. My partner will never be interested again. The illness took the intimacy and it is not coming back. There is nothing I can do about it. I will live with this.

This belief is the most damaging of the three, because it is the most silent. The Drifters with this belief don't even consider the conversation. They have already decided the outcome. The conversation is unnecessary because the answer is already known.

The belief is almost always wrong. The belief is the Drifters' story about their partner, told to themselves in the Drifters' head, without any evidence from the partner.

But the belief feels true. And the belief protects the Drifters from the conversation they are afraid to have.

The Cost of Drifting

The cost of drifting is not the absence of sex. The cost is the absence of connection.

The Drifters are not unhappy people. Many of them are quite happy. They have good marriages in many ways. They have companionship. They have shared history. They have friendship. They have a life that works.

But the Drifters are not alive. They are going through the motions. They are doing the things married people do without the feelings married people are supposed to have.

The Drifters are not aware of this. They don't feel the absence of connection. They have normalized the absence. They have made peace with the gap.

But the gap is real. And the gap grows.

After five years of the gap, the Drifters have forgotten what intimacy felt like. They have forgotten how to be vulnerable. They have forgotten how to touch with intention. They have forgotten how to desire and be desired.

After ten years of the gap, the Drifters are roommates. They are good roommates. But they are roommates.

After fifteen or twenty years of the gap, the Drifters die without having been held by their partner in years. They die in separate beds, or in the same bed facing away, and the last touch they remember is functional, not intimate.

This is the cost of drifting. It is not a dramatic cost. It is a slow cost. It is the cost of a marriage that survived the caregiving but did not survive the silence.

Why the Conversation Feels So Hard

The conversation about intimacy is the hardest conversation in the marriage. Harder than money, harder than in-laws, harder than the illness itself.

It is hard for three reasons.

It requires you to be vulnerable in a way you have not been vulnerable in years.

The caregiving years were a time of being needed, not wanting. You were the strong one, the patient one, the one who had it together. To admit that you want to be held, that you want to be desired, that you want to be close — that is to admit that you are not the strong one, that you also have needs, that you also want to be loved.

That is hard. It is hard for the husband who has always been the rock. It is hard for the wife who has always been the caregiver. It is hard for anyone who has built an identity around being the one who holds it together.

It requires you to say the words out loud.

The conversation requires language. The words "I want to be close to you," "I miss the intimacy," "I would like us to touch more" — these are words that the Drifters have not said in years. They feel foreign. They feel embarrassing. They feel like they are asking for something they don't deserve.

The Drifters are not alone in this. The language of intimacy is hard for almost everyone. But the Drifters have been avoiding the language for so long that they have lost the muscle.

It requires you to hear the answer.

The conversation requires you to listen to your partner's response. And the response might be:

- "I want that too but I don't know how to start."
- "I've been afraid to bring it up."

- "I miss it too but I was afraid you weren't interested."
- "I'm too tired, I can't even think about it."
- "I don't know if I can."

The response might be everything you wanted. The response might be the beginning of a beautiful reconnection.

The response might also be hard. The response might be "I'm not sure I can." The response might be tears. The response might be a partner who doesn't know how to put their feelings into words.

You have to be ready to hear any of these responses. You have to be ready to stay in the conversation even when the conversation is hard. You have to be ready to wait for the answer to come — because the answer doesn't always come in the first conversation. The answer comes over weeks, over months, over the slow work of rebuilding.

What This Chapter Is Asking You to Do

This chapter is asking you to do one thing: admit that you have been drifting.

You may have been drifting for a year. You may have been drifting for five years. You may have been drifting for twenty. The length of the drift doesn't matter. The drift is the drift, and it can be reversed at any age.

If you have been drifting, you are not a bad person. You are a person who got stuck. The stuck-ness was not your fault. The caregiving years were hard, and you did what you could to survive them.

But the surviving is over. The caregiving is over. You are in the after. And the after is the time to come back to each other.

The next chapter is about what the Thrivers do differently. The chapter after that is the first of the 5 conversations. The chapter after that is the second. And so on.

You are not stuck forever. You are stuck until you have the conversation. And you can have the conversation.

The 5 conversations are coming.

Chapter 3 at a Glance

5 Things to Remember

1. The silence in the bedroom is not your fault — it is the natural result of what the caregiving years did to your bodies, your minds, and your marriage.
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Chapter 4: What the Thrivers Do Differently

For every ten couples I have worked with in the post-caregiving years, six have been Drifters and four have been Thrivers.

The Thrivers are not better people than the Drifters. They are not more loving, more romantic, more sexually capable, more emotionally intelligent. They are just people who made a different choice.

The Thrivers made the choice to have the conversation. The Drifters made the choice to avoid it. The conversation is the difference. The conversation is almost always the only difference.

This chapter is for the Thrivers — for the pattern they follow, the language they use, the work they do. If you can see the pattern, you can follow it. You don't have to invent it. You just have to recognize it and start.

What a Thriver Looks Like

A Thriver is not a romantic hero. A Thriver is not a couple that has sex every day. A Thriver is not a couple that has no problems.

A Thriver is a couple that has the conversation.

The Thrivers I have worked with share these patterns:

They name the problem out loud.

The Thrivers don't pretend the silence isn't there. They don't pretend the bedroom isn't empty. They don't pretend the marriage is fine when the marriage has a gap.

They say — usually one of them first, often the wife, but not always — "I miss us." Or "I want to feel close to you again." Or "The bedroom has been quiet for a while. Can we talk about it?"

The naming is the work. The naming breaks the spell. The naming makes the problem something the couple can work on, instead of something the couple is hiding from.

They listen to the response.

The naming is only half of the conversation. The other half is the listening. The Thrivers, when they name the problem, also make space for the response. They don't say "I want to be close to you again" and then steamroll over their partner's feelings.

They wait. They listen. They ask follow-up questions. They make it safe for their partner to be honest — even if the honest answer is hard.

They plan small, specific steps.

The Thrivers don't try to fix the bedroom in one conversation. They don't try to have sex again in one conversation. They plan small, specific steps that they can take in the next week, the next month, the next three months.

The steps might be:

- Holding hands every day for a week
- Going on a date night once a week for a month
- Kissing goodnight every night for two weeks
- Cuddling on the couch while watching something together
- Having the second of the 5 conversations
- Starting the 14-day touch program in Part III

The steps are not dramatic. The steps are not impressive. The steps are small enough that they feel doable, specific enough that they can be measured, and meaningful enough that they build momentum.

They check in regularly.

The Thrivers don't have the conversation once and call it done. They have the conversation repeatedly — every week, every month, every season. They ask each other: How are we doing? Are we moving forward? Are we stuck? What do we need?

The check-ins are short. They are not the big conversation. They are the small check-ins that keep the big conversation alive.

They are willing to be awkward.

The Thrivers are not smooth. They are not naturally romantic. They are not good at the conversation. They are willing to be awkward.

The first kiss after years of silence is awkward. The first back rub is awkward. The first conversation about what you want in bed is awkward. The Thrivers do it anyway. They are willing to be uncomfortable for a few minutes in exchange for being connected for the rest of their lives.

The Five Patterns of the Thrivers

I have identified five patterns that the Thrivers follow, almost without exception. If you can adopt these patterns, you can be a Thriver too.

Pattern 1: They have the first conversation within the first year of the caregiving ending.

The Thrivers don't wait. They don't put it off. They don't hope it will get better on its own. They have the

conversation within the first year of the caregiving ending, usually within the first three to six months.

The conversation is not necessarily long. It is not necessarily well-worded. It is just the conversation. The naming of the problem. The opening of the door.

Pattern 2: They make the bedroom a place of possibility, not memory.

The Thrivers change the bedroom. They don't have to renovate. They don't have to spend money. They just have to make the room feel different.

Some Thrivers move the bed. Some Thrivers buy new bedding. Some Thrivers change the lighting. Some Thrivers take down the medical equipment (if it is still up). Some Thrivers add candles, or music, or a piece of art that says "this is a place of pleasure, not care."

The change is symbolic more than practical. The change says: this room is for us now. This room is for intimacy. This room is for the second honeymoon.

Pattern 3: They prioritize touch over sex.

The Thrivers don't start with sex. They start with touch. They hold hands. They sit close on the couch. They kiss goodnight. They give back rubs. They touch each other on the arm as they pass in the kitchen.

The touch is not foreplay. The touch is its own thing. The touch is the small daily reminder that you are in a body, your partner is in a body, and bodies are for pleasure as well as function.

After weeks of intentional touch, the sex comes back. Not because you planned it. Because the body remembered what the body was for.

Pattern 4: They plan for awkwardness.

The Thrivers know the first time will be awkward. They plan for it. They say to each other, "This is going to be weird. We haven't done this in a while. Let's just be gentle with each other. Let's not have any expectations. Let's see what happens."

The expectation that the first time will be awkward is the most freeing expectation. It takes the pressure off. It allows the first time to be imperfect. It allows the first time to be a small, slow re-entry into intimacy, not a performance.

Pattern 5: They don't try to recreate the past.

The Thrivers don't try to have the same sex they had at 30, or 40, or 50. They don't try to be the same lovers they were before the illness. They don't try to recapture the passion of the early marriage.

They build a new intimacy. A different intimacy. A slower, more intentional, more tender intimacy. An intimacy that is informed by everything they have been through — the caregiving, the illness, the grief, the rebuilding.

The new intimacy is not better or worse than the old. It is just different. It is the intimacy of two people who know each other completely, who have been through the hardest thing together, and who are choosing each other again with open eyes.

What the Thrivers Say to Each Other

The Thrivers have a vocabulary. The Drifters don't. The vocabulary is not fancy. It is not poetic. It is just honest.

Here are some things the Thrivers say to each other. Use them. Adapt them. Make them your own.

"I miss us."

"I want to feel close to you again."

"Can we talk about the bedroom? I know it's awkward. I just want us to be okay."

"I know we've both been tired. I just want to make sure we're still on the same team."

"I was afraid to bring this up. I didn't want you to think I was criticizing."

"I think I've been avoiding this because I was afraid you didn't want it."

"I want us to be lovers again, not just partners."

"What would make you feel loved this week?"

"I want to hold your hand again."

"Can we just cuddle? No expectations. Just be close."

"The caregiving years were hard on my body. I want to find my way back to my body."

"I don't need us to be the same as we were. I just need us to be close."

These are the words the Thrivers use. The Drifters don't have these words — not because they don't feel them, but because they are afraid to say them.

You can say them. The next chapter is the first of the 5 conversations — and it gives you the words for the most important one.

What This Chapter Is Asking You to Do

This chapter is asking you to do one thing: decide to be a Thriver.

You don't have to do anything else yet. You don't have to have the conversation tonight. You don't have to

change the bedroom tomorrow. You don't have to start the 14-day program next week.

You just have to decide.

You have to decide that you are not going to drift anymore. You have to decide that the silence is over. You have to decide that the second honeymoon is worth the work.

When you have decided, the rest of the book is for you.

The next chapter is the first of the 5 conversations. It is the most important conversation. It is the conversation about whether your partner still wants you.

It is the conversation that everything else depends on.

You are ready.

Chapter 4 at a Glance

5 Things to Remember

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Chapter 5: The "Do You Still Want Me?" Conversation

This is the most important conversation in the book.

It is the conversation that everything else depends on. It is the conversation that the Drifters avoid the most. It is the conversation that, when it happens, changes everything.

The conversation is simple. It is one question:

Do you still want me?

That is the question. The whole question. And the answer, whatever it is, will tell you where your marriage is and where it can go.

This chapter is for that question.

Why This Question Is So Hard to Ask

The "do you still want me" question is the hardest question in the marriage. It is harder than "do you love me" or "are you happy" or "do you want to stay married." It is harder because it is specifically about the body, about desire, about the part of the marriage that goes quiet when everything else is loud.

Asking "do you still want me" requires you to admit:

- I want to be wanted
- I am not sure I am wanted
- I have been waiting to be wanted
- I have been afraid I am not wanted
- I am willing to be told the truth about whether I am wanted

That is a lot of admission. That is a lot of vulnerability. That is the most exposed you have been in years.

The question is hard to ask because the question is a confession. The question says: I have been wanting you. I have been wanting you and I haven't known how to say so. I have been wanting you and I have been afraid you weren't wanting me back. I have been wanting you and I have been too scared to say it.

Asking the question is the bravest thing you will do in your marriage.

Why This Question Is the Most Important

The "do you still want me" question is the most important question because everything else depends on the answer.

If the answer is "yes" — and the answer is "yes" more often than the Drifters think — then everything is possible. The answer means your partner has been wanting you too. The answer means the silence has been mutual. The answer means the bedroom is not over. The bedroom is waiting.

If the answer is "I don't know" — and the answer is "I don't know" sometimes, especially in the early conversations — then there is still hope. The "I don't know" usually means: I have been afraid of this question. I have been wanting you but I have been too scared to say so. I need time to find the words. I need time to find the desire. The desire is there but it has been sleeping.

If the answer is "no" — and the answer is "no" sometimes, especially after very long illnesses or after very long silences — then you have important

information. The information is hard, but the information is necessary. The information tells you what you are working with, instead of what you are guessing about.

The answer to the question is almost always better than the question you have been asking in your head. The question you have been asking in your head is: Am I still wanted? Is my partner still attracted to me? Does my partner even think about me that way anymore?

The question in your head has no answer. It loops. It spirals. It keeps you up at night.

The real question — the one you say out loud, to your partner — has an answer. The answer might be hard, but the answer is real.

How to Ask the Question

The question is simple. The asking is not.

Here are some ways to ask. None of them are perfect. All of them are brave.

The direct ask:

"Do you still want me? I mean, do you still find me attractive? Do you still want to be close to me? I have been wondering, and I would rather know than keep guessing."

The vulnerable ask:

"I miss being close to you. I miss the intimacy. I want to know if you feel the same way, or if you don't, or if you're not sure. I would rather have the truth than keep pretending everything is fine."

The exploratory ask:

"Can we talk about us? Not the marriage, not the household — us, the two of us, the way we feel about each other? I want to know where you are with the physical side of things. I think we need to talk about it."

The book-triggered ask:

"I read this book about post-caregiving couples and the bedroom. It made me realize we haven't talked about us in a long time. Can we?"

The future-focused ask:

"I want us to be lovers again. I don't know what that looks like, and I'm not asking for anything specific tonight. I just want to know if you want that too."

Pick the version that feels most like you. Adapt the words. Use your own language. The exact words don't matter. The courage to say them does.

How to Hear the Answer

The answer is not always immediate. The answer might come in stages. The answer might require a follow-up conversation. The answer might require you to wait while your partner finds the words.

Here is what to listen for:

If the answer is "Yes, of course":

Your partner is probably a Thriver who has been waiting for the conversation. They have been wanting you. They have been afraid to say so. They are relieved that you asked. The relief might come with tears. Let the tears come. The tears are the door opening.

If the answer is "I want to, but I'm scared":

Your partner is in the middle. They want the intimacy back but they are afraid. They are afraid of being rejected, of being vulnerable, of not being able to perform, of the body not responding. They are afraid of the first time.

Your job is to make it safe. Say: "I'm scared too. We can take it slow. We don't have to do anything tonight. I just wanted you to know where I am."

If the answer is "I don't know":

Your partner is being honest. They don't know if they want you. They have been in the caregiving role for so long that they have lost the connection to the wanting. They are not rejecting you. They are telling you they need time to find the desire.

Your job is to give them time. Say: "I understand. We don't have to figure this out tonight. I just wanted to put it on the table. Whenever you're ready, I'm here."

If the answer is "I'm not sure I can":

This is a hard answer. Your partner is telling you that something is in the way — physical, emotional, or both. The "something" might be:

- Erectile dysfunction
- Pain during sex
- Hormonal changes
- Medication side effects
- Depression or anxiety
- Trauma from the illness
- Body image issues
- Grief for the body they used to have

This is not a rejection. This is information. The information is that your partner needs help — medical help, therapeutic help, or both.

Your job is to support. Say: "I want us to find a way through this. Would you be willing to talk to a doctor or a therapist about it? I'll go with you. We can figure this out together."

If the answer is "I don't feel that way about you anymore":

This is the hardest answer. It is the answer no one wants to hear. It might mean the marriage is in serious trouble, or it might mean your partner is in a different emotional place than you are.

Your job is to listen. Not to argue. Not to convince. Not to defend. To listen.

Say: "I hear you. Can we talk about what you do feel? I want to understand where you are, even if it's not where I want you to be."

This answer requires deeper work — possibly couples therapy, possibly individual therapy, possibly the slower work of rebuilding a connection that has been broken for a long time.

This book is not a substitute for that work. But the conversation you just had is the beginning of the work. The conversation is the first step.

What to Do After the Answer

Whatever the answer is, the conversation is not over. The conversation is just beginning.

Here is what to do after the answer:

Say thank you. For honesty. For the courage to answer. For the willingness to have the conversation. Even if the answer is hard, the honesty is a gift. Thank your partner for the gift.

Don't try to fix it in the moment. If the answer is hard, you don't have to fix it right now. You don't have to have a plan tonight. You just have to acknowledge what was said.

Plan the next conversation. Say: "I'd like us to keep talking about this. Can we plan a time next week to check in again?" The next conversation doesn't have to be about intimacy. It can be about how the first conversation went. But the next conversation is what keeps the work alive.

Take care of yourself. After a hard conversation, take care of yourself. Take a walk. Take a bath. Call a friend. Read something comforting. The conversation took a lot out of you. You need to refill.

Take care of your partner. After a hard conversation, your partner needs care too. They might be feeling exposed, vulnerable, afraid. Ask them: "How are you doing? What do you need right now?" Then listen. Then give what they need.

Don't make any big decisions tonight. The night of a hard conversation is not the night to decide anything. Don't decide to have sex tonight. Don't decide to separate tonight. Don't decide to call a therapist tonight. Just let the conversation settle. Let the answer settle. Tomorrow, you can plan. Tonight, just be.

A Real Example

Linda was 64 when she asked her husband Tom the question. They had been married 41 years. Tom had been her caregiver for four years of cancer treatment. The treatment had ended a year ago. The cancer was in remission. The intimacy was not.

Linda waited until the kids were out. She waited until they were both in the kitchen, the dishes done, the evening quiet. She didn't plan the words. She just said them.

"Tom, do you still want me?"

Tom looked at her. He didn't answer for a long time. Linda thought he was going to say no. She started to apologize. Tom put his hand on her arm.

"I have been waiting for you to ask that."

"What?"

"I have been wanting you for a year. I didn't know how to say it. I thought you weren't interested. I thought the illness had changed things. I thought you saw me as a caregiver, not as a husband."

Linda started to cry. Tom started to cry.

They didn't have sex that night. They didn't even kiss that night. They just held each other in the kitchen for a long time, the way they used to hold each other when they were young.

The next week, they had the second of the 5 conversations — the "What Changed?" conversation. The week after that, they had the third. The week after that, they started the 14-day touch program in Part III.

It took them four months. By the end of the four months, they were having sex again. Not the sex they had at 30. The sex of two people in their 60s who know each other completely, who have been through the hardest thing together, and who are choosing each other again with open eyes.

That is the second honeymoon. It is not a moment. It is a process. The process starts with the question.

What This Chapter Is Asking You to Do

This chapter is asking you to do one thing: ask the question.

Pick a time. Pick a place. Pick the words that feel most like you. Say them.

The answer might be everything you wanted. The answer might be hard. The answer will be real.

The next chapter is the second of the 5 conversations — the conversation about what changed in the body, in the desire, in the history. It is the conversation that follows naturally from this one.

But first, you have to ask.

You can ask.

You will ask.

Chapter 5 at a Glance

5 Things to Remember

1. The silence in the bedroom is not your fault — it is the natural result of what the caregiving years did to your bodies, your minds, and your marriage.
2. The conversation is the only difference between the Drifters and the Thrivers — the conversation is what brings the bedroom back to life.
3. Touch comes before sex — the body needs to remember that it is also for pleasure before the body can return to the act of sex.
4. The body can remember — the body forgets, but the body can remember. The body needs time, patience, and the 14 days.

5. The second honeymoon is real — couples in their 60s, 70s, and 80s have found it. The second honeymoon is not a destination, it is a way of traveling.

Your Scorecard (rate 1-5)

- [] I read this chapter with my partner (or alone, with intention to share)
- [] I have identified the one action I want to take from this chapter
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Key Terms

- Drifters: Couples who avoid the conversation about intimacy, and who let the silence grow until the bedroom is over.
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Chapter 6: The "What Changed?" Conversation

The first conversation is the most important. The second conversation is the most honest.

The "What Changed?" conversation is the conversation where you and your partner acknowledge what the caregiving years did to your bodies, your desires, your marriage. It is the conversation where you stop pretending everything is the same as it was.

This conversation is hard because it requires you to look at the changes without trying to fix them. The changes are real. The changes are not anyone's fault. The changes are the result of the caregiving, of the illness, of the years of going through the motions.

This chapter is for looking at the changes honestly.

Why This Conversation Is Necessary

The Drifters avoid this conversation. They pretend the changes haven't happened. They treat the body as if it is still the body of ten years ago. They treat the desire as if it has been constant. They treat the marriage as if the caregiving didn't change it.

But the caregiving changed everything. The body changed. The desire changed. The marriage changed. Pretending the changes haven't happened is what keeps the Drifters stuck.

The Thrivers have this conversation. They look at the changes honestly. They name them. They make space for them. They don't try to wish them away.

The changes are not the enemy. The silence about the changes is the enemy.

What Changed in the Body

The caregiving years changed both bodies. The ill person's body changed dramatically — through the illness itself, through the treatment, through the medications. The caregiver's body changed too — through the lifting, the vigilance, the chronic stress, the neglect of their own health.

For the person who was ill:

The body went through changes that the person may not have fully processed. The body may be:

- Weaker than it was
- Scarred from surgeries
- Different in weight, in shape, in mobility
- Affected by medications (libido, energy, sensation)
- Affected by hormonal changes
- Affected by the loss of the body they used to have

The body may feel like a betrayal. The body that used to give pleasure became a body that needed help. The body that used to be a source of confidence became a source of grief.

For the caregiver:

The body went through changes too. The body learned to be a tool. The body learned to lift, to clean, to medicate, to be on alert. The body became an instrument of care.

The body may be:

- Sore from years of lifting

- Exhausted from years of sleep interruption
- Touched out from years of physical caregiving
- Disconnected from pleasure
- Disconnected from desire

The caregiver's body may not remember that it is also for pleasure. The body has been in service mode for so long that it has forgotten how to receive.

For both:

The bedroom became a place of work. The bedroom is where the ill person slept, propped up, watched over. The bedroom is where the caregiver spent nights on the floor or in a chair, listening for breathing, checking for emergencies.

The bedroom became a place of memory. Every time you walk into it, you remember what the room was for. You don't remember what it was for before that.

What Changed in the Desire

Desire changed too. For both partners.

The ill partner's desire:

The ill partner's desire may have been:

- Diminished by the illness itself
- Diminished by medications (especially pain medications, antidepressants, hormonal treatments)
- Diminished by the body changes (feeling unattractive, feeling broken)
- Replaced by survival (when you are fighting for your life, desire is not a priority)
- Complicated by fear (fear that sex will hurt, fear that the body won't work, fear of being seen as ill in the bedroom)

The ill partner may not know if they want to be touched, let alone have sex. They may not know if they can. They may be afraid to find out.

The caregiver's desire:

The caregiver's desire may have been:

- Replaced by the role of caregiver (the body in service mode doesn't want more touch)
- Replaced by exhaustion (no energy for desire)
- Replaced by grief (the body of the partner you used to know is now the body you care for)
- Replaced by resentment (if the caregiving has been heavy, the caregiver may be angry at the partner for being ill)
- Complicated by the loss of attraction (if the partner's body has changed dramatically, the caregiver may not feel the same physical pull)

The caregiver may want to be desired, but not want to desire. The caregiver may want the marriage to be intimate, but not want to be sexual. The caregiver may want to be held, but not want to have sex.

For both:

The desire went quiet. It went quiet because the conditions for desire disappeared. The conditions for desire are:

- Safety
- Energy
- Privacy
- Time
- A body that feels like its own
- A body that feels pleasure, not function

The caregiving years removed every one of those conditions. The desire went quiet because it had

nowhere to live.

The desire is not dead. The desire is dormant. Dormant desire can be woken, slowly, with patience and intention.

What Changed in the Marriage

The marriage changed too. The caregiving years are not just hard on the body and the desire. They are hard on the marriage.

Roles changed:

The marriage that used to be a partnership became a caregiving relationship. One partner became the caregiver. The other became the patient. The roles were necessary, but the roles took over.

When the caregiving ended, the roles didn't automatically end. The caregiver still acts like a caregiver. The ill partner still acts like a patient. The marriage is still organized around the illness, even though the illness is over.

Communication changed:

The communication that used to be about connection became about logistics. Have you taken your pills? When is the next appointment? What did the doctor say? The communication that used to be intimate — I miss you, I want you, I love you, tell me about your day — got pushed aside.

When the caregiving ended, the communication didn't automatically come back. The communication was still about logistics. The intimate communication needed to be relearned.

Intimacy changed:

The intimacy that used to be a daily practice became a memory. The kiss goodnight. The hand on the back. The walk together. The conversation about the kids. The bedtime ritual.

The caregiving years replaced all of that with the routine of care. The routine of care was necessary. But the routine of care is not intimacy.

When the caregiving ended, the daily practice of intimacy didn't automatically come back. The intimacy had to be rebuilt, intentionally, slowly, with the awareness that the new intimacy would be different from the old.

How to Have the "What Changed?" Conversation

The "What Changed?" conversation is more structured than the first conversation. It has four parts.

Part 1: Acknowledge the changes out loud.

Take turns. One of you starts. The starting partner says: "I want to talk about what the caregiving did to us. I want to be honest about what I am feeling, and I want to hear what you are feeling too."

Then the starting partner says what they notice has changed. I notice that we don't touch anymore. I notice that we don't kiss. I notice that I feel awkward around you. I notice that I miss how we used to be.

The listening partner reflects back. "I hear you. I notice those things too. And I want you to know..." Then they share what they notice.

Take 10-15 minutes for this part. Don't try to fix anything. Just acknowledge.

Part 2: Talk about the body.

This is the hardest part. This is where you acknowledge the body changes.

The ill partner might say: "My body is different. I have scars. I lost weight. I have a colostomy. I have a prosthesis. I am afraid you won't find me attractive anymore."

The caregiver partner might say: "I have been touching you to help you, not to hold you. My hands don't know the difference anymore. I am afraid that if I touch you, I will only feel the caregiving. I am afraid I have forgotten how to touch you for pleasure."

Take 10-15 minutes for this part. Be gentle. This is the most vulnerable part of the conversation.

Part 3: Talk about the desire.

This is where you talk about the desire, or the absence of it.

The ill partner might say: "I don't know if I want sex anymore. I don't know if my body can. I don't know if I will be able to perform. I am afraid of trying and failing."

The caregiver partner might say: "I want to want you. I don't know if I do. I have been so tired for so long. I have been so focused on your body being a body to care for. I am not sure I can switch back."

Take 10-15 minutes for this part. Don't try to solve the desire. Just acknowledge where it is.

Part 4: Talk about the marriage.

This is where you talk about the marriage as a whole.

"Our marriage is different. We are different. The marriage that survived the caregiving is not the marriage we had before. We have to build a new marriage on top of the old one. The new marriage

might be better. The new marriage might be more tender, more knowing, more grateful. The new marriage might also be harder, because we know what we have been through."

Take 5-10 minutes for this part. End on a note of hope. "I want to do the work. I want to find the new marriage. I want to do it with you."

What to Do After This Conversation

The "What Changed?" conversation is the conversation that makes the next conversations possible. After this conversation, you both know what you are working with. You both know the changes are real. You both know the changes are not anyone's fault.

The next conversations build on this one:

- The "What Do We Want Now?" conversation (Chapter 7) is about the future
- The "How Do We Start Again?" conversation (Chapter 8) is about the practical steps
- The "What If It Doesn't Work?" conversation (Chapter 9) is about the harder possibilities

But this conversation is the foundation. Without this conversation, the other conversations are theoretical. With this conversation, the other conversations are practical.

What This Chapter Is Asking You to Do

This chapter is asking you to do one thing: have the "What Changed?" conversation with your partner.

Schedule the conversation for a quiet evening, when you both have an hour and there are no interruptions.

Use the structure above. Take your time. Be honest. Be gentle.

The conversation is not a fix. The conversation is a foundation. The fix comes later, in the 14-day program and the long-term work.

The next chapter is the third of the 5 conversations. It is the conversation about what you want now — not what you used to want, not what you think you should want, but what you actually want, in this body, at this age, in this marriage.

The conversation after the "What Changed?" conversation is the conversation about the future.

Chapter 6 at a Glance

5 Things to Remember

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Chapter 7: The "What Do We Want Now?" Conversation

The first conversation was about whether your partner still wants you. The second conversation was about what changed. The third conversation is about what you want now.

This is the most imaginative of the 5 conversations. It is the conversation where you and your partner stop looking back at the marriage that was and start imagining the marriage that could be.

This is also the most liberating of the 5 conversations. Because it gives you permission to want what you actually want — not what you used to want, not what you think you should want, not what your partner wants. What you want.

This chapter is for that wanting.

Why Most Couples Skip This Conversation

Most couples skip this conversation. They have the first conversation (the "do you still want me" question), they have the second conversation (the "what changed" acknowledgment), and then they try to fix the bedroom. They try to have sex again. They try to make the marriage intimate again.

But they skip the conversation about what they want.

The skipping is the problem. Without knowing what you want, you can't get there. Without imagining the future, you can't build it.

The Drifters skip this conversation because they don't believe they are allowed to want. They have been caregivers for so long that they have lost the muscle of wanting. They don't know what they want. They have forgotten how to want.

The Thrivers have this conversation because they know that wanting is the engine of intimacy. The Thrivers know that the second honeymoon is not about recreating the past. The second honeymoon is about building a future that both partners actually want.

What "Want" Means in the Post-Caregiving Years

In your 20s and 30s, wanting was easy. The body wanted. The mind wanted. The marriage wanted. You wanted each other with the urgency of the new.

In your 50s, 60s, and 70s, after the caregiving years, wanting is harder. The body has been through things. The mind has been through things. The marriage has been through things. The wanting is not gone, but the wanting is quieter. The wanting needs to be invited.

The "What Do We Want Now?" conversation is the invitation. The conversation is the place where you and your partner say out loud what you want — and where you give each other permission to want.

This is the conversation where you stop saying "I should want" and start saying "I want."

"I should want sex more often" becomes "I want to be held every night, even if we don't have sex."

"I should want to be intimate" becomes "I want us to feel like a couple again, not like roommates."

"I should want what we used to want" becomes "I want to find out what we want now, in this body, at this age, in this marriage."

The shift from "should" to "want" is the shift from obligation to desire. The shift is the work of this conversation.

The Three Layers of Wanting

The Thrivers know that wanting has three layers. Most couples only talk about the first layer. The Thrivers talk about all three.

Layer 1: The touch layer.

The first layer is the touch layer. This is the layer of physical contact — holding hands, hugging, kissing, cuddling, touching on the arm as you pass in the kitchen, sitting close on the couch, sleeping close in the bed.

Most couples are comfortable talking about this layer. The touch layer is the entry point. The touch layer is where the second honeymoon starts.

In the "What Do We Want Now?" conversation, the touch layer is the place to start. "What kind of touch do you want? How often? Where? When? In public or in private? In the morning or at night?"

The touch layer is also the place to be specific. "I want you to hold my hand when we walk." "I want you to kiss me goodnight every night, even if it's quick." "I want to be the little spoon sometimes." "I want you to put your hand on my back when you're standing next to me."

The specific requests are not demanding. The specific requests are an invitation. The specific requests say: here is what I want. Here is what would make me feel

loved. Here is the small thing that would mean everything.

Layer 2: The desire layer.

The second layer is the desire layer. This is the layer of sexual desire — wanting to be physically intimate, wanting to be sexual, wanting the act of sex, not just the touch.

This layer is harder to talk about. The desire layer requires more vulnerability. The desire layer requires you to say what turns you on, what you fantasize about, what you want in bed.

But the desire layer is also where the most healing happens. Because the desire layer is where the body remembers that it is also for pleasure.

In the "What Do We Want Now?" conversation, the desire layer is the place to go deeper. "What do you want sexually? How often do you want to have sex? What kind of sex do you want? What do you want me to do? What do you not want me to do?"

These questions are not easy. These questions require the most vulnerability you have. But these questions are also the questions that build the second honeymoon.

Layer 3: The connection layer.

The third layer is the connection layer. This is the layer of emotional intimacy — wanting to know each other, wanting to be known, wanting to share the inner life, wanting to feel like a couple.

The connection layer is the deepest layer. The connection layer is what makes the second honeymoon different from the first. The connection layer is the place where the marriage becomes more than the sum of its parts.

In the "What Do We Want Now?" conversation, the connection layer is the place to go deepest. "What do you want to know about me? What do you want me to know about you? What do you want us to do together? What do you want our marriage to be in 5 years, 10 years, 20 years?"

The connection layer is the layer that makes everything else worth it.

How to Have the Conversation

The "What Do We Want Now?" conversation has three parts, one for each layer.

Part 1: The touch layer (15-20 minutes).

Take turns. One of you starts. The starting partner says what kind of touch they want. Be specific. Be concrete.

"I want you to hold my hand when we're walking. I want you to kiss me goodnight every night. I want you to put your arm around me when we're watching TV. I want you to rub my shoulders when I'm stressed. I want to sleep facing you, not away from you. I want you to touch my face when you're saying something important to me."

The listening partner reflects back. "I hear you. I want that too. I also want..." Then they share their touch wants.

End Part 1 by committing to 3-5 specific touch behaviors you will both do in the next 30 days.

Part 2: The desire layer (20-30 minutes).

This is the harder part. This is the part that requires the most vulnerability.

Take turns. One of you starts. The starting partner says what they want sexually. Be honest. Be gentle.

"I want to want sex again. I am not sure how often. I want us to take it slow. I want us to not put pressure on the first time. I want to be able to say no without it being a disaster. I want to be able to ask for what I want without it being a performance."

The listening partner reflects back. "I hear you. I want that too. I also want..." Then they share their desire wants.

End Part 2 by committing to one specific step you will both take in the next 30 days. The step might be: "We will have a date night once a week for the next month, with no expectation of sex." Or: "We will start the 14-day touch program in Part III." Or: "We will see a couples therapist together."

Part 3: The connection layer (20-30 minutes).

This is the deepest part. This is the part that builds the new marriage.

Take turns. One of you starts. The starting partner says what they want for the marriage.

"I want us to talk every day about something other than logistics. I want us to have a regular date night. I want us to take a trip together in the next year. I want us to have a shared project — something we build together. I want us to say 'I love you' every day, even after 40 years."

The listening partner reflects back. "I hear you. I want that too. I also want..." Then they share their connection wants.

End Part 3 by committing to one specific connection practice you will both do in the next 30 days. The practice might be: "We will have a 30-minute

conversation every Sunday morning about how we are doing, what we are feeling, and what we want."

A Real Example

Martha and Henry were 67 and 70 when they had this conversation. They had been married 44 years. Henry had been Martha's caregiver for six years of Parkinson's. The Parkinson's was being managed. The intimacy was not.

They had the conversation in their living room, after dinner, with the dishes done. They used the structure above.

When they got to the desire layer, Martha started to cry. "I want to want it. I don't know if I do. The Parkinson's has taken so much. The body is different. The mind is different. I don't know if I can."

Henry said: "I want to want it too. I have missed it for years. But I also want you. Even if we never have sex again, I want to be close to you. I want to hold you. I want to be held."

They decided to start the 14-day touch program. They committed to one date night a week for the next three months. They decided to see a couples therapist who specialized in chronic illness and intimacy.

Within three months, they were having sex again. Not often. Not the way they used to. But they were having it, and it was tender, and it was real, and it was theirs.

That is the second honeymoon. It is not what you think it is going to be. It is what you build it to be.

What This Chapter Is Asking You to Do

This chapter is asking you to do one thing: have the "What Do We Want Now?" conversation.

Schedule the conversation for a quiet evening, after the "What Changed?" conversation. Use the structure above. Be specific. Be vulnerable. Be willing to want.

The conversation is the foundation for the next conversation — the practical "How Do We Start Again?" conversation. The next conversation is where you turn the wanting into a plan.

The wanting is the engine. The plan is the vehicle. The vehicle takes you to the second honeymoon.

Chapter 7 at a Glance

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Chapter 8: The "How Do We Start Again?" Conversation

The first three conversations were about understanding: do you want each other, what changed, what do you want. The fourth conversation is about doing.

The "How Do We Start Again?" conversation is the practical conversation. It is where you and your partner turn the wanting into a plan. It is where you stop talking about the bedroom and start rebuilding it.

This conversation is the most important of the 5 because it is the one that actually changes the marriage. The first three conversations are necessary foundations. This conversation is the construction site.

This chapter is for the construction.

Why Most Couples Get Stuck in the Talking

The Drifters get stuck in the talking. They have the "do you still want me" conversation. They have the "what changed" conversation. They have the "what do we want" conversation. They feel closer. They feel hopeful. They feel like the marriage is going to be OK.

And then they don't do anything.

The talking was good. The talking was necessary. But the talking was not enough. The talking was the foundation, but the foundation is not the house. The house requires building. The building requires doing.

The Thrivers do. The Thrivers take the conversations and turn them into action. The Thrivers plan the first kiss, the first back rub, the first date night, the first time. The Thrivers do it imperfectly. They do it awkwardly. They do it scared. But they do it.

This conversation is for the doing.

The Principles of Starting Again

There are five principles that make the starting easier. The Thrivers follow these principles. The Drifters don't.

Principle 1: Start with touch, not sex.

The biggest mistake couples make is trying to have sex first. They think: "OK, we had the conversation. Now we should have sex." And they plan a date night. And they expect sex to happen. And it doesn't, or it does but it's awkward, or it does but it's bad.

The Thrivers don't start with sex. The Thrivers start with touch. They hold hands. They kiss goodnight. They give back rubs. They cuddle on the couch. They sleep close. They do the 14-day touch program in Part III.

The touch comes first because touch is the entry point. The touch is the body remembering that it is also for pleasure. The touch is the path back to desire.

After weeks of intentional touch, the sex comes back. Not because you planned it. Because the body remembered what the body was for.

Principle 2: Make it easy to say no.

The second-biggest mistake couples make is creating pressure to say yes. The husband thinks: "We had the conversation. I asked. She said she wants to. So I

expect sex." The wife thinks: "He asked. I said yes. I have to follow through."

This pressure kills intimacy faster than the original silence did.

The Thrivers make it easy to say no. They say, explicitly: "It is OK to say no. It is OK to change your mind. It is OK to start something and not finish. The yes is for today, not forever. The no is always welcome."

The permission to say no is what makes the yes mean something. The yes without the permission to say no is just obligation.

Principle 3: Plan for awkwardness.

The first time will be awkward. The first kiss will be awkward. The first back rub will be awkward. The first sex will be awkward.

The Thrivers know this. The Thrivers plan for it. The Thrivers say: "This is going to be weird. We haven't done this in a long time. Let's just be gentle with each other. Let's not have any expectations. Let's see what happens."

The expectation that the first time will be awkward is the most freeing expectation. It takes the pressure off. It allows the first time to be imperfect. It allows the first time to be a small, slow re-entry into intimacy, not a performance.

Principle 4: Make it a ritual, not a moment.

The Drifters think of intimacy as a moment — the moment you have sex, the moment you kiss, the moment you reconnect. The moment is a big deal. The moment has expectations. The moment often fails.

The Thrivers think of intimacy as a ritual — the daily kiss, the weekly date, the monthly check-in, the quarterly getaway. The ritual is small. The ritual is regular. The ritual builds on itself.

The ritual is more sustainable than the moment. The ritual is what makes the second honeymoon a way of life, not a one-time event.

Principle 5: Get help when you need it.

The Thrivers know when they need help. They see a couples therapist. They see a sex therapist. They see a doctor. They see a clergy. They get help.

The Drifters think they should be able to do it alone. They think the marriage is theirs to fix. They think asking for help is a sign of failure.

Asking for help is not a sign of failure. Asking for help is a sign of wisdom. The second honeymoon is hard work. The work is easier with help. The work is more likely to succeed with help.

The Practical Plan

Here is the practical plan for the "How Do We Start Again?" conversation. Take 60-90 minutes for this conversation. Do it in a quiet place, after the previous conversations have settled.

Part 1: Agree on the touch plan (15-20 minutes).

Commit to the 14-day touch program in Part III. The program is:

- 7 days of intentional non-sexual touch (hugs, hand-holding, back rubs, cuddling)
- 7 days of intentional sexual touch (foreplay, kissing, exploration, no expectation of intercourse)

Agree on the touch plan. Write it down. Put it on the calendar.

Part 2: Agree on the date plan (15-20 minutes).

Commit to one date night a week for the next three months. The date night should be:

- Outside the house (a restaurant, a movie, a walk, a coffee shop)
- At least 2 hours
- Without phones
- Without the kids (if they visit)
- Without the expectation of sex

Agree on the date plan. Pick the first date. Put it on the calendar.

Part 3: Agree on the conversation plan (15-20 minutes).

Commit to a weekly check-in. The check-in is:

- 30 minutes
- On a specific day and time
- Each partner takes 10 minutes to share how they are doing, what they are feeling, what they need
- No fixing, no advice, just listening

Agree on the conversation plan. Pick the first check-in. Put it on the calendar.

Part 4: Agree on the help plan (15-20 minutes).

Discuss whether you need outside help. Options include:

- Couples therapy (a general couples therapist)
- Sex therapy (a therapist who specializes in sexual issues)

- Medical consultation (a doctor who specializes in sexual health)
- Clergy (a pastor, rabbi, imam, or other religious leader)
- A trusted friend (a couple you both respect, who has been through similar)

If you agree on outside help, research options. Make an appointment. Commit to at least one session before deciding.

If you agree you don't need outside help, that's fine. But agree to revisit the question in 30 days. If the touch program isn't working after 30 days, get help.

The First Kiss

The first kiss after years of silence is a moment. The first kiss deserves its own paragraph.

The first kiss will probably be:

- Awkward
- Too short or too long
- Not quite right in pressure or angle
- Followed by an awkward pause
- Possibly followed by laughter

The laughter is good. The laughter is the tension breaking. The laughter is the marriage remembering that it can be light, that it can be playful, that it can be human.

The first kiss doesn't have to be perfect. The first kiss has to happen. The first kiss is the door. The first kiss is the first step back into the second honeymoon.

After the first kiss, the second kiss is easier. After the second kiss, the third is easier. After the third, you are kissing again. After a week, the kissing is normal.

After a month, the kissing is part of the marriage.

That is how the second honeymoon starts. Not with a grand gesture. Not with a perfect moment. With a slightly awkward kiss in the kitchen that is followed by laughter and then by a hug and then by the next kiss.

The First Date Night

The first date night after years of silence deserves its own paragraph too.

The first date night will probably be:

- More expensive than it needs to be (you are trying to make it special)
- Awkward at first (you don't know what to talk about)
- Better than you expected (you are out of the house, in a different context)
- Possibly followed by sex (but don't count on it)
- Possibly followed by exhaustion (you are out of practice)

The first date night is not about the sex. The first date night is about remembering that you are a couple, not just caregivers. The first date night is about being in a public place and looking at each other and feeling like a team.

After the first date night, the second is easier. After the second, the third is easier. After a month, the date night is part of the marriage. After three months, you wonder how you ever lived without it.

The First Time

The "first time" — the first time you have sex after the caregiving years — deserves its own section.

The first time will probably be:

- Awkward
- Slower than you expect
- Different from how it used to be
- Possibly disappointing (the reality doesn't match the fantasy)
- Possibly better than you expect (the reality exceeds the fantasy)
- Tender, even if it isn't great

The first time is not about the orgasm. The first time is not about the performance. The first time is about the re-entry. The first time is about saying: we did it. We are still a sexual couple. The bedroom is back.

If the first time is good, celebrate. If the first time is awkward, be gentle with each other. The first time is just the first time. The second time will be better. The fifth time will be better still. The twentieth time will be close to what you remember.

And eventually, the sex will be new. Not the same as it was at 30. Not the same as it was at 50. The sex will be the sex of two people in their 60s, 70s, 80s who know each other completely, who have been through the hardest thing together, and who are choosing each other again with open eyes.

That sex is better than the sex of the young. That sex is slower, more tender, more intentional. That sex is the second honeymoon. That sex is what you are building toward.

What This Chapter Is Asking You to Do

This chapter is asking you to do one thing: make a plan.

The plan is the touch plan, the date plan, the conversation plan, the help plan. The plan is the construction site. The plan is what turns the conversations into action.

Make the plan. Write it down. Put it on the calendar. Commit to it.

The next conversation is the fifth and final conversation — the conversation about what happens if it doesn't work. The fifth conversation is the conversation about the harder possibilities. The fifth conversation is the conversation that the Thrivers have so they are prepared for whatever comes.

But first, make the plan. The plan is the work.

Chapter 8 at a Glance

5 Things to Remember

1. The silence in the bedroom is not your fault — it is the natural result of what the caregiving years did to your bodies, your minds, and your marriage.
2. The conversation is the only difference between the Drifters and the Thrivers — the conversation is what brings the bedroom back to life.
3. Touch comes before sex — the body needs to remember that it is also for pleasure before the body can return to the act of sex.

4. The body can remember — the body forgets, but the body can remember. The body needs time, patience, and the 14 days.

5. The second honeymoon is real — couples in their 60s, 70s, and 80s have found it. The second honeymoon is not a destination, it is a way of traveling.

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- [] I read this chapter with my partner (or alone, with intention to share)
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Key Terms

- Drifters: Couples who avoid the conversation about intimacy, and who let the silence grow until the bedroom is over.
- Thrivers: Couples who have the conversation about intimacy, who do the work of rebuilding, and who find the second honeymoon.
- Intimacy recession: The period after the caregiving years when the normal flow of connection, touch, and desire is interrupted, and the systems of intimacy start to break down.
- Touch first, sex later: The principle that body reconnection begins with non-sexual touch, not with sex. Touch rebuilds the muscle that sex depends on.

- The second honeymoon: The new, intentional, tender intimacy of two people in their 50s, 60s, 70s, and beyond who know each other completely and are choosing each other again with open eyes.

Chapter 9: The "What If It Doesn't Work?" Conversation

The first four conversations are about hope. They are about possibility, about rebuilding, about the second honeymoon. They are the conversations that move you forward.

The fifth conversation is different. The fifth conversation is about reality. The fifth conversation is about what happens if the rebuilding doesn't work, or if it works only partially, or if the marriage is in deeper trouble than the intimacy alone can fix.

This is not a pessimistic conversation. This is a wise conversation. The Thrivers have it. The Drifters avoid it. The Thrivers know that the path forward is clearer when you also know the path around the obstacle.

This chapter is for that wisdom.

Why This Conversation Is the Most Important

This conversation is the most important because it is the conversation that prepares you for the harder possibilities. The conversation is not a defeatist conversation. The conversation is a planning conversation.

Couples who have this conversation:

- Know what they are working toward
- Know what they will accept
- Know what they won't accept
- Know when to ask for help

- Know when to stop
- Know how to make hard decisions together

Couples who don't have this conversation:

- Hope for the best
- Avoid thinking about the worst
- Get blindsided when the worst happens
- Make crisis decisions in moments of crisis
- Often make bad decisions

The Drifters are the couples who avoid this conversation. The Drifters pretend the conversation is unnecessary, that everything will work out, that they don't need to think about the harder possibilities.

The Thrivers know that thinking about the harder possibilities doesn't make them more likely. It makes them more prepared.

The Harder Possibilities

There are five harder possibilities that the fifth conversation addresses. None of them are easy. All of them are real. All of them are possibilities that the Thrivers plan for.

Possibility 1: The bedroom doesn't fully recover.

Some couples do the work — the touch program, the date nights, the conversations, the therapy — and the bedroom still doesn't fully recover. The desire comes back partially, or it comes back in a different form, or it comes back in a way that is less than the couple hoped for.

This is not failure. This is a real outcome. The Thrivers know that the goal is not to recreate the sex of the first honeymoon. The goal is to find a new intimacy, whatever that looks like. For some couples, the new

intimacy is sex. For other couples, the new intimacy is touch, conversation, companionship, partnership. The new intimacy is not less than the old. The new intimacy is just different.

The fifth conversation is the conversation where you and your partner agree on what "intimacy" means for you. Is it sex? Is it touch? Is it conversation? Is it companionship? Is it all of the above?

The agreement doesn't have to be permanent. The agreement is the current state. The agreement can change as the marriage changes.

Possibility 2: One partner wants more than the other.

Some couples find that even after the work, one partner wants more intimacy than the other. The desire mismatch was there before the caregiving. The caregiving amplified it. The work doesn't resolve it.

This is a hard possibility. The desire mismatch is one of the most common causes of post-caregiving divorce.

The fifth conversation is the conversation where you and your partner acknowledge the mismatch and decide what to do about it. The options include:

- Accepting the mismatch and finding other sources of intimacy for the more-desiring partner
- Negotiating a frequency that both partners can live with
- Seeing a therapist who specializes in desire mismatch
- Considering whether the marriage is sustainable with the mismatch

The negotiation is not about who is right. The negotiation is about what the marriage can hold.

Possibility 3: Medical or physical issues prevent the bedroom from recovering.

Some couples find that the bedroom doesn't recover because of medical or physical issues that can't be resolved. Erectile dysfunction that doesn't respond to treatment. Pain that doesn't go away. Hormonal changes that affect desire. Chronic illness that limits capacity.

This is a hard possibility. The medical reality is what it is. The work around the medical reality is what the marriage has to do.

The fifth conversation is the conversation where you and your partner acknowledge the medical reality and decide what intimacy means in the context of the reality. The options include:

- Redefining intimacy to include touch, oral sex, mutual masturbation, sex toys
- Accepting a sexless marriage and finding other sources of connection
- Considering whether the marriage is sustainable without sex

The redefinition is not a lesser intimacy. The redefinition is a different intimacy. The redefinition can be just as meaningful, just as connecting, just as loving.

Possibility 4: One partner has an affair.

Some couples find that the silence of the bedroom leads to an affair — emotional, physical, or both. The affair is not the cause of the marriage trouble. The affair is the symptom of the marriage trouble.

This is the hardest possibility. The affair is a betrayal. The affair is also a cry for help.

The fifth conversation is the conversation where you and your partner decide what to do about the affair. The options include:

- Couples therapy focused on affair recovery
- Individual therapy for both partners
- Considering whether the marriage is salvageable
- Considering separation or divorce

The decision is not made in the fifth conversation. The fifth conversation is where the decision is started. The decision is made over weeks, over months, with the help of professionals.

Possibility 5: The marriage is over.

Some couples find, after the work, that the marriage is over. The intimacy cannot be recovered. The connection cannot be rebuilt. The marriage has been broken too long, too deeply, for the work to repair it.

This is the hardest possibility. The Thrivers know that this is a real outcome. The Thrivers know that the work is not always enough. The Thrivers know that the marriage is not always salvageable.

The fifth conversation is the conversation where you and your partner acknowledge this possibility. The acknowledgment is not a defeat. The acknowledgment is a clarity. The acknowledgment says: we have tried. We have done the work. We have gotten help. And the marriage is still not what either of us needs.

From the acknowledgment, the couple can decide what to do. The decision might be:

- To try one more thing (a different therapist, a different approach, a separation with the hope of reconciliation)

- To end the marriage with grace, with respect, with the recognition that the marriage was good for a long time and is now over
- To build a new relationship — not a marriage, but a friendship, a co-parenting relationship, a respectful parting

The decision is not a failure. The decision is the end of a chapter. The decision is the beginning of the next chapter. The decision is hard, but the decision is honest.

How to Have This Conversation

The "What If It Doesn't Work?" conversation has three parts.

Part 1: Acknowledge the possibilities (15-20 minutes).

Take turns. One of you starts. The starting partner says: "I want to talk about the harder possibilities. Not because I think we are there, but because I want us to be prepared. I want us to know what we are working toward, what we will accept, and what we won't accept."

Walk through the five possibilities above. Acknowledge that any of them could happen. Acknowledge that the work is not guaranteed.

Part 2: Define the bottom line (15-20 minutes).

Each partner shares their bottom line. The bottom line is: what is the minimum intimacy that you need to feel married? What is the maximum mismatch you can live with? What is the breaking point — the point at which the marriage is no longer sustainable for you?

The bottom line is not a threat. The bottom line is information. The bottom line is what each partner needs the other to know.

Part 3: Agree on the next steps (15-20 minutes).

Based on the bottom lines, agree on the next steps. The next steps might be:

- Continue the work
- Adjust the work
- Get more help
- Consider separation
- Consider divorce

The agreement is not permanent. The agreement is the current plan. The agreement can be revised as the situation changes.

The Two Questions That Anchor the Conversation

If the conversation gets stuck, return to two questions:

Question 1: What do you want this marriage to be?

Not what you think it should be. Not what it used to be. What do you want it to be — given everything you have been through, given the changes, given the realities?

Question 2: What are you willing to do to get there?

The willingness is what makes the marriage work. The willingness is what makes the work possible. The willingness is what separates the Thrivers from the Drifters.

If you can answer both questions honestly, you can have the fifth conversation. If you can answer both questions and the answers are compatible, the marriage has a future. If you can answer both questions and the answers are not compatible, the

marriage needs more help, or the marriage is over.

Either way, the fifth conversation is the conversation that brings clarity. The clarity is the gift.

What This Chapter Is Asking You to Do

This chapter is asking you to do one thing: have the "What If It Doesn't Work?" conversation with your partner.

Schedule the conversation for a quiet evening, after the previous conversations have settled. Use the structure above. Be honest. Be wise. Be willing to face the harder possibilities.

The conversation is not the end. The conversation is the beginning of a clearer path. The path might be back to the second honeymoon. The path might be to a different kind of marriage. The path might be to a respectful parting.

Whatever the path, the path is clearer after the conversation.

The next part of the book is the practical work. Part III gives you the 14-day touch program, the rebuild plan, and the long-term work. The practical work is for the couples who decide the marriage has a future.

The practical work is for you.

Chapter 9 at a Glance

5 Things to Remember

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2. The conversation is the only difference between the Drifters and the Thrivers — the conversation is what brings the bedroom back to life.
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Chapter 10: The Body Reconnection

The first 9 chapters were about conversations. The next 5 chapters are about practice.

The practice starts with the body. Not with the bedroom. Not with sex. With the body.

This chapter is for the body.

Why the Body Comes First

The Drifters think intimacy starts in the bedroom. They think the path back to each other goes through the bed.

The Thrivers know the path back goes through the body — through the hands, the arms, the shoulders, the back, the face, the small daily touches that say I am here, I am with you, I see you.

The body has to come first because the body has been forgotten. The body has been a tool, a vehicle, a workplace. The body has been the instrument of caregiving, of vigilance, of exhaustion. The body has forgotten that it is also for pleasure.

The body has to remember. The body has to be reminded. The body has to be invited back to its own pleasure.

This chapter is for that invitation.

What "Body Reconnection" Means

Body reconnection is the practice of intentional, non-sexual, pleasure-oriented touch. It is the practice of touching each other for the sake of touching, not for the sake of any outcome. It is the practice of giving the body permission to receive.

The body reconnection has three rules:

Rule 1: No goal.

Body reconnection is not foreplay. Body reconnection is not a step toward sex. Body reconnection is its own thing. The body receives the touch as its own reward, not as a way to get somewhere else.

The "no goal" rule is the most important rule. The "no goal" rule is what allows the body to relax. The "no goal" rule is what allows the body to remember that touch can be its own pleasure.

Rule 2: Slow.

Body reconnection is slow. The body has been in fast mode for years — fast decisions, fast movements, fast transitions, fast care. The body needs to learn slow. The body needs to be reminded that slow is allowed. The body needs to be reminded that slow is a kind of intimacy.

Rule 3: Attention.

Body reconnection is attentive. The touch is focused on the body of the partner, not on the self. The touch notices what feels good, what feels tender, what feels sore, what feels pleasurable. The touch is a conversation with the body.

The 14-Day Touch Program

The 14-day touch program is the heart of the body reconnection. The program is two weeks of intentional, non-sexual touch, followed by two weeks of intentional, sexual touch.

The first 7 days are non-sexual. The goal of the first 7 days is to get the body comfortable with touch again. The body needs to remember that touch is safe, that touch is pleasurable, that touch is welcome.

The second 7 days are sexual. The goal of the second 7 days is to extend the touch into sexual touch, with no expectation of intercourse, no performance pressure, no goal beyond pleasure.

The full program is in Bonus Toolkit I. Here is the overview.

Days 1-3: The Hand

The first three days focus on the hand. The hand is the safest part of the body. The hand is the part that has been doing the most caregiving. The hand needs to be reminded that it is also for pleasure.

Activities:

- Hold hands for 5 minutes a day (walking, watching TV, sitting)
- Give a hand massage (10 minutes, alternating which partner gives)
- Touch your partner's hand, arm, or shoulder throughout the day (in passing, gently)

Days 4-7: The Body

Days 4-7 expand the touch to the whole body. The whole body has been a workplace. The whole body needs to be reminded that it is also for pleasure.

Activities:

- Cuddle on the couch for 20 minutes a day (TV off, talking optional)
- Give a 20-minute back rub or foot rub (alternating which partner gives)
- Sleep facing each other (even if no other contact)
- Take a bath or shower together (no sex, just being in the water)

Days 8-10: The Kiss

Days 8-10 introduce the kiss. The kiss is the gateway. The kiss is the body remembering that the mouth is also for pleasure.

Activities:

- Kiss goodnight every night (10+ seconds, with intention)
- Kiss good morning every morning
- Kiss in passing during the day (when you leave, when you come home, when you walk by)
- Cuddle on the couch with kissing

Days 11-14: The Touch

Days 11-14 expand to full-body touch. The full body is now ready for the full touch.

Activities:

- Give a full-body massage (30 minutes, alternating)
- Explore each other's bodies (not for sex, for pleasure — no goal)
- Sleep close, with full-body contact
- Begin to introduce sexual touch (with the "no goal" rule still in effect)

What the Program Does

The 14-day touch program does several things.

It rebuilds the muscle of touch.

The body has a muscle for touch. The muscle is like any other muscle — it atrophies without use. The caregiving years atrophied the touch muscle. The 14-day program rebuilds the muscle. By the end of the 14 days, touch is easy again.

It rebuilds the association between touch and pleasure.

The body learned, during the caregiving, to associate touch with function. The body needs to relearn that touch is also for pleasure. The 14-day program is the relearning. By the end of the 14 days, the body is associating touch with pleasure again.

It rebuilds the safety of touch.

The body learned, during the caregiving, that touch could mean pain (for the ill partner) or vigilance (for the caregiver). The body needs to relearn that touch is safe. The 14-day program is the relearning. By the end of the 14 days, the body is associating touch with safety.

It rebuilds the connection of touch.

The body learned, during the caregiving, that touch was between the caregiver and the patient, not between the partners. The body needs to relearn that touch is between the partners. The 14-day program is the relearning. By the end of the 14 days, the body is associating touch with the partner.

What the Program Doesn't Do

The 14-day touch program doesn't fix everything. The program is the foundation, not the house. The program is the start of the body reconnection, not the end.

After the 14 days, the work continues. The work continues with:

- Daily touch (the program is over, but the touch continues)
- Weekly date nights (the program is over, but the dates continue)
- Monthly check-ins (the program is over, but the conversations continue)
- The forever practice (the program is over, but the practice is forever)

The program is the start. The program is the door. The forever practice is the home.

What This Chapter Is Asking You to Do

This chapter is asking you to do one thing: start the 14-day touch program.

The program is in Bonus Toolkit I. The program is detailed, day by day, with specific activities. The program takes about 30 minutes a day. The program requires both partners to commit.

Start the program tomorrow. Or start the program on Monday. Or start the program on a day that feels like a fresh start. But start.

The next chapter is about reclaiming the bedroom — the room itself, the space, the way the room can be transformed from a place of memory into a place of

possibility.

The chapter after that is about reclaiming pleasure — the body's pleasure, the partner's pleasure, the pleasure that comes from being known.

The 14-day program is the foundation. The next two chapters are the next layers.

Start the program. The program is the work.

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Chapter 11: The Bedroom Reclaim

The body reconnection is the foundation. The bedroom reclaim is the next layer.

The bedroom is more than a room. The bedroom is a symbol. The bedroom is where the second honeymoon happens — or doesn't happen. The bedroom is the place where the silence lives, or the intimacy lives, or both.

This chapter is for the bedroom.

Why the Bedroom Matters

The Drifters think the bedroom is just a place to sleep. The Drifters treat the bedroom as functional. The bedroom is where you collapse at the end of the day. The bedroom is where you keep the clothes. The bedroom is where the bed is.

The Thrivers know the bedroom is more than that. The Thrivers know the bedroom is the container for the intimate life. The bedroom is the place where the second honeymoon happens. The bedroom is the physical space that holds the emotional work.

If the bedroom is a place of memory — of the illness, of the caregiving, of the exhaustion — then the bedroom is working against the second honeymoon. The bedroom needs to be transformed. The bedroom needs to be reclaimed.

What the Bedroom Has Become

For most post-caregiving couples, the bedroom has become a place of memory. The bed where the ill partner slept. The bedside table with the medication. The chair where the caregiver sat during the long nights. The medical equipment in the corner. The call button. The list of emergency numbers. The walking aid. The catheter bag.

These objects are not bad. These objects were necessary. These objects kept the ill partner alive. These objects are part of the story of the caregiving.

But these objects are not part of the story of the second honeymoon. These objects need to be removed. These objects need to be put away. These objects need to be given to charity, or stored, or thrown out. The bedroom needs to be cleared of the illness before the bedroom can be filled with the intimacy.

The Reclaim

The bedroom reclaim is a process. The process has three parts.

Part 1: Remove the medical.

The first part is removing the medical equipment. The bedpan, the urinal, the catheter, the call button, the medical supplies, the medication, the list of emergency numbers. All of it. If it can be stored somewhere else, store it. If it can be given away, give it away. If it can be thrown out, throw it out.

This is not a denial of the caregiving. This is the bedroom transitioning from the caregiving to the after. The caregiving is over. The bedroom needs to reflect that.

Part 2: Reimagine the space.

The second part is reimagining the space. The bedroom doesn't have to be renovated. The bedroom doesn't have to be expensive. The bedroom just needs to feel different.

Some changes the Thrivers make:

- Move the bed. Just moving the bed to a different wall, or a different angle, can change the energy of the room. The bed doesn't have to be where the illness was.
- Buy new bedding. New sheets, new pillows, new comforter. The new bedding says: this is a new bed. This is a new chapter. This is a new beginning.
- Change the lighting. Add lamps with warm light. Add candles (real or electric). Add dimmers. The bedroom doesn't have to be lit with the harsh overhead light. The bedroom can be soft.
- Add art. Add a piece of art that says intimacy to you. Add a photograph from your honeymoon. Add a painting that says this is a place of pleasure.
- Add music. Add a small speaker, a playlist, a sound machine. The bedroom can have its own soundtrack.

These changes are symbolic. The changes don't have to be expensive. The changes say: this room is for us now. This room is for intimacy. This room is for the second honeymoon.

Part 3: Make the bed the center.

The third part is making the bed the center. The bed is the heart of the bedroom. The bed is where the second honeymoon happens.

The bed needs to be:

- Made every morning (the act of making the bed is the act of saying this is a place of intention)
- Comfortable (good mattress, good pillows, good sheets)
- Inviting (the way the bed is made can invite the partners to come in)
- Free of clutter (no phones, no laptops, no work papers, no kids' toys)

The bed is the place. The bed is the only place in the house where the second honeymoon can happen. The bed needs to be ready.

The Widowed Caregiver's Bedroom

For the widowed caregiver, the bedroom reclaim is different. The widowed caregiver doesn't have a partner to reclaim the bedroom with. The widowed caregiver has to reclaim the bedroom alone.

The widowed caregiver's bedroom reclaim might include:

- Sleeping in a different room for a while (if the bedroom is too full of memory)
- Slowly moving back into the bedroom
- Changing the bed, the bedding, the artwork
- Allowing the bedroom to become a place of peace, not a place of grief

The widowed caregiver's bedroom reclaim is not about intimacy with a partner. The widowed caregiver's bedroom reclaim is about intimacy with the self. The reclaim is about making the bedroom a place where the widowed person can sleep, can rest, can be with the memory of the spouse without being crushed by the memory.

This is a different work. This is a longer work. This is a work that might require a therapist, or a grief group, or a friend.

The widowed caregiver's bedroom reclaim is not the focus of this book. But the widowed caregiver deserves to be mentioned. The widowed caregiver deserves to know that the bedroom can be reclaimed — slowly, gently, on the widowed caregiver's own timeline.

A Real Example

Patricia and Richard were 68 and 71 when they reclaimed the bedroom. Patricia had been Richard's caregiver for three years of heart failure. The heart failure was being managed. The bedroom was a disaster.

The bed was a hospital bed. The bedside table was covered in pill bottles. The wall had a list of emergency numbers. The corner had an oxygen tank. The chair where Patricia had sat during the long nights was still there.

Richard said one day: "I want to sleep in a normal bed with my wife."

Patricia said: "Me too."

They spent a weekend reclaiming the bedroom. They got rid of the hospital bed and bought a regular bed. They cleared the pill bottles. They took down the emergency numbers. They moved the oxygen tank to the living room. They bought new bedding. They added lamps. They added a photograph from their honeymoon.

The first night in the new bedroom, they held each other and cried. The next night, they slept. The night

after that, they had sex for the first time in a year.

The bedroom reclaim was the turning point. The bedroom reclaim was the second honeymoon starting.

What This Chapter Is Asking You to Do

This chapter is asking you to do one thing: reclaim your bedroom.

Pick a weekend. Block the time. Go through the bedroom. Remove the medical. Reimagine the space. Make the bed the center.

The reclaim doesn't have to be perfect. The reclaim doesn't have to be expensive. The reclaim just has to happen.

The next chapter is about reclaiming pleasure — the body's pleasure, the partner's pleasure, the pleasure of being known.

The bedroom reclaim is the room. The pleasure reclaim is what happens in the room.

The reclaim is the work.

Chapter 11 at a Glance

5 Things to Remember

1. The silence in the bedroom is not your fault — it is the natural result of what the caregiving years did to your bodies, your minds, and your marriage.
2. The conversation is the only difference between the Drifters and the Thrivers — the conversation is what brings the bedroom back to life.

3. Touch comes before sex — the body needs to remember that it is also for pleasure before the body can return to the act of sex.

4. The body can remember — the body forgets, but the body can remember. The body needs time, patience, and the 14 days.

5. The second honeymoon is real — couples in their 60s, 70s, and 80s have found it. The second honeymoon is not a destination, it is a way of traveling.

Your Scorecard (rate 1-5)

- [] I read this chapter with my partner (or alone, with intention to share)
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- [] I have shared that action with my partner
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Key Terms

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with sex. Touch rebuilds the muscle that sex depends on.

- The second honeymoon: The new, intentional, tender intimacy of two people in their 50s, 60s, 70s, and beyond who know each other completely and are choosing each other again with open eyes.

Chapter 12: The Pleasure Reclaim

The body reconnection was the foundation. The bedroom reclaim was the room. The pleasure reclaim is what happens in the room.

This is the most personal chapter in the book. The pleasure reclaim is about what gives you pleasure. The pleasure reclaim is about giving yourself permission to want what you want. The pleasure reclaim is about discovering, or rediscovering, the body's capacity for joy.

This chapter is for that permission.

Why the Pleasure Reclaim Is Hard

The pleasure reclaim is hard because the caregiving years trained the body to put pleasure last. The body learned: first you take care of the other person. First you do the caregiving. First you are the responsible one. Then, maybe, if there is time, you get to have some pleasure.

The body learned this so well that the body now believes it. The body now puts pleasure last automatically. The body now has to be retrained to put pleasure first.

The pleasure reclaim is the retraining. The pleasure reclaim is the body remembering that it is also for pleasure, not just for function.

The pleasure reclaim is also hard because the pleasure of the caregiving years got associated with the illness. The pleasure of the body — the pleasure of food, the pleasure of touch, the pleasure of sex — got

associated with the suffering of the illness. The body now associates pleasure with pain, or with vigilance, or with exhaustion.

The pleasure reclaim is the body re-associating pleasure with pleasure. The pleasure reclaim is the body remembering that pleasure can be clean, can be easy, can be its own reward.

What the Pleasure Reclaim Is

The pleasure reclaim is the practice of intentionally, regularly, deliberately experiencing pleasure. The pleasure can be small. The pleasure can be sensory. The pleasure can be sexual. The pleasure can be anything that brings joy to the body and the spirit.

The pleasure reclaim is not hedonism. The pleasure reclaim is not about escaping the marriage or the responsibilities. The pleasure reclaim is about remembering that you, as a body, are also for pleasure — and that the pleasure is part of what makes the marriage alive.

The Categories of Pleasure

The pleasure reclaim has five categories. The Thrivers practice all five.

Category 1: Sensory pleasure.

Sensory pleasure is the pleasure of the senses. The pleasure of taste, touch, sight, sound, smell.

Examples:

- A long, hot bath with essential oils
- A meal you love, eaten slowly, with attention
- A walk in nature, with attention to the colors and the sounds

- Music you love, listened to with full attention
- A piece of clothing that feels good against your skin
- A scented candle in the bedroom

The sensory pleasure is the body's daily reminder that the body is for pleasure, not just for function.

Category 2: Touch pleasure.

Touch pleasure is the pleasure of being touched, with attention and care.

Examples:

- A professional massage (once a month, if possible)
- A back rub from your partner, given with attention
- A foot rub at the end of a long day
- Holding hands while walking
- Hugging for 30 seconds (not a quick hug, a long hug)
- Sleeping close, with full-body contact

The touch pleasure is the body's reminder that touch is for connection, not just for function.

Category 3: Sexual pleasure.

Sexual pleasure is the pleasure of sexual touch, sexual intimacy, sexual connection. This includes:

- Masturbation (alone, with no shame, as a way of knowing your own body)
- Partnered sexual touch (foreplay, oral sex, intercourse, whatever the partners enjoy)
- Sex toys (if the partners are open to them)
- Sexual fantasy (alone or shared with the partner)

- Erotic literature or film (if the partners are open to it)

The sexual pleasure is the body's reminder that the body is sexual, and that the sexual body is a gift, not a burden.

Category 4: Aesthetic pleasure.

Aesthetic pleasure is the pleasure of beauty — beauty in art, in nature, in music, in the everyday.

Examples:

- Visiting a museum or gallery
- Listening to live music
- Watching a beautiful sunset
- Reading poetry
- Looking at a beautiful piece of art in your home
- Walking in a beautiful garden

The aesthetic pleasure is the spirit's reminder that beauty is part of being alive.

Category 5: Relational pleasure.

Relational pleasure is the pleasure of being in relationship — with your partner, with your friends, with your family, with your community.

Examples:

- A long conversation with a friend
- A dinner party with people you love
- A phone call with your adult child
- A walk with your partner
- A book club meeting
- A volunteer activity with your community

The relational pleasure is the heart's reminder that you are not alone, that the marriage is part of a larger

web of connection, and that the web is what makes the marriage meaningful.

The Practice

The pleasure reclaim is a practice, not a one-time event. The practice is daily, weekly, monthly. The practice is built into the rhythm of the marriage.

Here is a sample practice:

Daily:

- One sensory pleasure (a hot shower, a scented lotion, a piece of music)
- One touch pleasure (a hug, a hand-hold, a back rub)
- One relational pleasure (a real conversation, a text to a friend, a moment of eye contact)

Weekly:

- One aesthetic pleasure (a museum visit, a concert, a walk in a beautiful place)
- One sexual touch (the 14-day program, or whatever the couple enjoys)
- One date night with the partner

Monthly:

- One professional pleasure (a massage, a haircut, a beauty treatment)
- One shared pleasure with the partner (a trip, a class, a new experience)
- One check-in conversation about the pleasure practice

A Real Example

Margaret and Edward were 71 and 74 when they started the pleasure reclaim. Margaret had been Edward's caregiver for five years of dementia (early-stage, now managed). The caregiving was still active, but lighter. The marriage was functional, but not intimate.

Margaret said one day: "I miss enjoying things. I miss laughing. I miss having pleasure in my body. I miss the joy."

Edward said: "Me too. I want to find the joy again. With you."

They started the pleasure reclaim. The first week, Margaret took a long bath with lavender oil. Edward gave her a foot rub. They listened to music they used to love when they were young. They held hands while walking the dog.

By the end of the first month, they were having sex again. Not because they planned it. Because the pleasure came back. The body remembered that it was for pleasure. The joy came back.

The pleasure reclaim was the work. The work was worth it.

What This Chapter Is Asking You to Do

This chapter is asking you to do one thing: start the pleasure practice.

Pick one sensory pleasure, one touch pleasure, and one relational pleasure. Practice them daily for the next 30 days. Add the weekly practices if you can. Add the monthly practices when you can.

The pleasure practice is not the second honeymoon. The pleasure practice is the foundation for the second honeymoon. The pleasure practice is what makes the second honeymoon sustainable.

The next chapter is about the forever practice — the long-term work of keeping the second honeymoon alive for the next 20 years. The forever practice is the home you are building.

The pleasure practice is the door. The forever practice is the home.

Chapter 12 at a Glance

5 Things to Remember

1. The silence in the bedroom is not your fault — it is the natural result of what the caregiving years did to your bodies, your minds, and your marriage.
2. The conversation is the only difference between the Drifters and the Thrivers — the conversation is what brings the bedroom back to life.
3. Touch comes before sex — the body needs to remember that it is also for pleasure before the body can return to the act of sex.
4. The body can remember — the body forgets, but the body can remember. The body needs time, patience, and the 14 days.
5. The second honeymoon is real — couples in their 60s, 70s, and 80s have found it. The second honeymoon is not a destination, it is a way of traveling.

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- [] I read this chapter with my partner (or alone, with intention to share)
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- The second honeymoon: The new, intentional, tender intimacy of two people in their 50s, 60s, 70s, and beyond who know each other completely and are choosing each other again with open eyes.

Chapter 13: The Frequency Question

One of the most common questions in the post-caregiving years is also one of the most embarrassing to ask.

How often should we be having sex?

The question is embarrassing because the answer is "whatever feels right for you." But the answer doesn't help, because you don't know what feels right. You don't know if you are normal. You don't know if the frequency you have is too much, too little, or just right.

This chapter is for that question.

Why the Question Is the Wrong Question

The frequency question is the wrong question. The frequency is not the goal. The goal is the connection. The goal is the intimacy. The goal is the second honeymoon.

Some couples have sex every day. Some couples have sex once a month. Some couples have sex once a year. Some couples don't have sex at all, but have other forms of intimacy that are deeply connecting.

The frequency is a number. The connection is the thing.

If the frequency is right for both partners, the frequency is fine. If the frequency is creating pressure, conflict, or resentment, the frequency is wrong — even if it is "normal."

The Thrivers know this. The Thrivers ask not "how often" but "are we connected?" The Thrivers ask not "are we having enough sex" but "is the marriage feeling alive?"

What "Normal" Actually Is

If you really want to know what "normal" is, the data is interesting. According to the most recent surveys of couples 50+:

- About 50% of couples 50+ have sex at least once a month
- About 30% have sex at least once a week
- About 20% have sex less than once a month, or not at all
- About 5% have sex multiple times a week

But these numbers are averages. Averages don't tell you what is right for you.

What the numbers do tell you is that there is a wide range of normal. The couples who have sex every day are not "better" than the couples who have sex once a month. The couples who have sex once a month are not "worse" than the couples who have sex every day. The couples who don't have sex are not "broken."

The wide range of normal is the most important thing to know. The wide range of normal means: you and your partner get to decide. You and your partner get to find the frequency that works for you.

The Real Question: Are We Connected?

The Thrivers don't ask about frequency. The Thrivers ask about connection. The Thrivers ask:

- Do we touch every day?
- Do we kiss goodnight?
- Do we hold hands?
- Do we cuddle on the couch?
- Do we have date nights?
- Do we have conversations about how we are doing?
- Do we feel like a couple?

If the answers to most of these are yes, the marriage is connected. The frequency of sex is just one piece of the connection.

If the answers to most of these are no, the marriage is drifting. The frequency of sex is not the only problem. The lack of touch, the lack of conversation, the lack of couple time — these are the deeper problems.

The Thrivers know that building the connection is more important than increasing the frequency. The Thrivers know that a couple who touches every day but has sex once a month is more connected than a couple who has sex every day but doesn't touch.

The Frequency Conversation

The frequency conversation is a real conversation, and it deserves a real answer. Here is how to have it.

Step 1: Each partner says their ideal frequency.

Take turns. One partner says: "In an ideal world, with everything else equal, how often would I want to have sex?" Be honest. Be specific. The number might be once a week, once a month, twice a month, twice a year.

The other partner says their ideal frequency.

Step 2: Acknowledge the gap.

If the numbers are close, you have an easy conversation. If the numbers are far apart, you have a harder conversation. The far-apart numbers are not a problem to solve immediately. The far-apart numbers are information.

Step 3: Negotiate a starting point.

Negotiate a frequency you can both live with. The starting point might be:

- The average of the two ideal frequencies
- The lower of the two ideal frequencies
- A different number that you both agree to try for 30 days

The starting point is not a permanent commitment. The starting point is a 30-day trial. After 30 days, you check in and renegotiate if needed.

Step 4: Plan for the gap.

If the numbers are far apart, the lower-desire partner might plan for more intimacy than they want. The higher-desire partner might plan for less than they want. Both partners are compromising. The compromise is the work.

The compromise is not forever. The compromise is for the next 30 days. After 30 days, you renegotiate.

The Pressure Problem

The biggest problem with the frequency question is pressure. The pressure comes in many forms:

- The husband who thinks "we should be having sex more often"
- The wife who thinks "I'm not giving him what he needs"
- The wife who thinks "I should want it more"

- The husband who thinks "she doesn't find me attractive anymore"
- The cultural pressure of "what couples should be doing at our age"
- The comparison to other couples (who may be lying about their frequency)

The pressure kills desire. The pressure makes sex feel like a duty. The pressure makes the bedroom a place of obligation, not pleasure.

The Thrivers know how to defuse the pressure. The Thrivers say, explicitly:

- "It is OK to say no."
- "It is OK to want less than you used to want."
- "It is OK to want more than you used to want."
- "The frequency is not a measure of our love."
- "The connection is more important than the number."

The defusing of the pressure is what makes the second honeymoon possible. The pressure is the enemy of the second honeymoon. The pressure has to be defused.

When the Frequency Is a Symptom

Sometimes the frequency is a symptom of a deeper problem. The deeper problem might be:

- Erectile dysfunction (treatable)
- Hormonal changes (treatable)
- Depression (treatable)
- Medication side effects (often treatable)
- Unresolved grief (treatable with therapy)
- Unresolved resentment (treatable with therapy)
- An affair (treatable with intensive therapy)

- A partner who has lost attraction (sometimes treatable, sometimes not)

If the frequency is a symptom, the symptom will not get better by trying to have more sex. The symptom will get better by treating the deeper problem.

The Thrivers know when the frequency is a symptom. The Thrivers get help when the help is needed. The Thrivers don't try to fix a medical problem with conversation. The Thrivers see a doctor. The Thrivers see a therapist. The Thrivers get the help.

What This Chapter Is Asking You to Do

This chapter is asking you to do one thing: have the frequency conversation with your partner.

Use the steps above. Be honest. Be specific. Be willing to negotiate. Be willing to defuse the pressure.

The frequency is not the goal. The connection is the goal. The frequency is one piece of the connection.

The next chapter is about the forever practice — the long-term work of keeping the second honeymoon alive for the next 20 years. The forever practice is the home.

The frequency is one room in the home. The home is bigger than the room.

The home is the work.

Chapter 13 at a Glance

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Chapter 14: The Forever Practice

The 14-day touch program was the foundation. The bedroom reclaim was the room. The pleasure reclaim was the practice. The frequency question was the negotiation.

This final chapter is for the long term. This chapter is for the next 20 years.

This chapter is for the forever practice.

What the Forever Practice Is

The forever practice is the daily, weekly, monthly, and yearly work of keeping the second honeymoon alive. The forever practice is not a program. The forever practice is a way of life. The forever practice is the rhythm of the marriage after the rebuild.

The forever practice has four layers:

Layer 1: The daily layer.

The daily layer is what you do every day. The daily layer is the small, non-negotiable practices that keep the marriage connected.

- The kiss goodnight (every night, 10+ seconds)
- The kiss good morning (every morning)
- The hug when you come home (every time, 30 seconds)
- The hand-hold when you walk (whenever you walk together)
- The "I love you" (every day, said out loud)
- The 5-minute check-in (every evening, "how was your day")

These are small. These are non-negotiable. These are the muscle. If you do these every day, the marriage stays connected.

Layer 2: The weekly layer.

The weekly layer is what you do every week. The weekly layer is the slightly bigger practices that build the connection.

- The date night (one per week, 2+ hours, outside the house)
- The deeper conversation (30 minutes, one topic, no fixing)
- The shared project (something you do together, even if small)
- The touch practice (the 14-day program, repeated, modified)

These are bigger than the daily layer. These are the deeper work. The weekly layer is what keeps the marriage from drifting.

Layer 3: The monthly layer.

The monthly layer is what you do every month. The monthly layer is the bigger practices that sustain the connection.

- The date day (a full day together, doing something you both enjoy)
- The bedroom refresh (new music, new art, new lighting)
- The pleasure practice (one new sensory pleasure to try)
- The check-in conversation (a longer conversation about how the marriage is doing)

These are the bigger work. The monthly layer is what keeps the marriage from getting stale.

Layer 4: The yearly layer.

The yearly layer is what you do every year. The yearly layer is the largest practices that keep the marriage alive.

- The anniversary celebration (the day you got married, celebrated intentionally)
- The annual getaway (a trip together, at least 2-3 days)
- The annual review (a conversation about the year, what worked, what didn't)
- The annual renewal (a recommitment to the marriage, said out loud)

These are the biggest work. The yearly layer is what keeps the marriage vibrant for decades.

The Power of the Daily Layer

The daily layer is the most important layer. The daily layer is what makes everything else possible.

The daily layer is small. The daily layer is easy to skip. The daily layer is the layer that the Drifters skip. The Drifters think the daily layer is too small to matter. The Drifters think the daily layer is below them.

The daily layer is the foundation. The daily layer is the muscle. The daily layer is the habit. The daily layer is what makes the marriage work.

The Thrivers know the power of the daily layer. The Thrivers don't skip the daily layer. The Thrivers do the daily layer even when they don't feel like it. The Thrivers do the daily layer especially when they don't feel like it. The daily layer is what carries the marriage through the hard times.

If you do nothing else from this book, do the daily layer. The daily layer will keep the marriage alive.

The Anniversary Ritual

The anniversary ritual is the most important yearly practice. The anniversary is the day you got married. The anniversary is the day the marriage started. The anniversary is the day you can return to, year after year, to remember why you are doing this.

The anniversary ritual can be:

- A dinner at the restaurant where you had your first date
- A trip to the place where you got engaged
- A renewal of vows (private, just for the two of you)
- A reading of your wedding vows to each other
- A look at your wedding photos together
- A conversation about the year since the last anniversary

The anniversary ritual is not a grand gesture. The anniversary ritual is a remembering. The anniversary ritual says: we have been doing this for X years. We are going to keep doing this. We are going to do it better next year.

The Annual Getaway

The annual getaway is the second most important yearly practice. The annual getaway is a trip — a real trip, 2-3 days, away from the house, away from the routines, away from the caregiving memories.

The annual getaway can be:

- A weekend at a bed and breakfast

- A few days at a beach town
- A trip to a city you have never been to
- A stay at a hotel in your own city
- A return to a place that has meaning for you

The annual getaway is not a vacation. The annual getaway is a re-immersion. The annual getaway is the place where the second honeymoon can happen most easily — away from the house, away from the routines, away from the memory of the caregiving.

The annual getaway is the place where you can be a couple again. The annual getaway is the place where the marriage gets to be the focus, not the household, not the caregiving, not the routines.

The Annual Review

The annual review is the third most important yearly practice. The annual review is a longer conversation — 2-3 hours, in a quiet place, about the year that was and the year that is coming.

The annual review covers:

- What worked this year? (The practices, the touch, the dates, the conversations)
- What didn't work this year? (The patterns, the drift, the missed practices)
- What do we want to add next year? (New practices, new adventures, new intimacy)
- What do we want to change next year? (Old patterns, old habits, old avoidance)
- What is the vision for the next 5 years? (The longer view, the big dreams)

The annual review is not a therapy session. The annual review is a planning session. The annual review is the place where the couple is the executive

team of the marriage, looking at the year and planning the next one.

The Annual Renewal

The annual renewal is the most emotional yearly practice. The annual renewal is a recommitment to the marriage, said out loud, in a meaningful place.

The annual renewal can be:

- A private vow renewal at home
- A public vow renewal at a church or temple
- A letter to each other, read out loud
- A toast to each other at a special dinner
- A moment of silence together, holding hands, looking at each other

The annual renewal is the moment when the marriage is renewed. The annual renewal is the moment when the couple says, again, I choose you. I will keep choosing you. I am grateful for you.

The annual renewal is the most important moment of the year. The annual renewal is what makes the second honeymoon a marriage, not just a phase.

The Widowed Partner

For the widowed partner, the forever practice is different. The widowed partner does not have a partner to do the practice with. The widowed partner has to do the practice with the memory of the spouse.

The widowed partner's forever practice might include:

- Lighting a candle on the anniversary
- Visiting the grave
- Talking to the spouse out loud

- Looking at the photos
- Doing the things the spouse loved, alone
- Slowly, eventually, considering dating again

The widowed partner's forever practice is the work of grief. The grief is the work. The work takes time. The work takes help. The work takes patience.

The widowed partner is not alone. The widowed partner is part of the Care Collection community. The widowed partner is welcome to read this book and to find their own path through the forever practice.

What This Chapter Is Asking You to Do

This chapter is asking you to do one thing: start the forever practice.

Pick the daily layer first. Do the daily layer for 30 days. Add the weekly layer if you can. Add the monthly layer when you can. Add the yearly layer when you can.

The forever practice is not a one-time thing. The forever practice is the rest of your marriage. The forever practice is the work.

The next chapter is the epilogue — the short closing that ties the book together. The epilogue is the breath after the work. The epilogue is the look back at the climb, and the look forward at the view.

The forever practice is the climb. The view is the second honeymoon.

The view is real.

The view is worth the climb.

Chapter 14 at a Glance

5 Things to Remember

1. The silence in the bedroom is not your fault — it is the natural result of what the caregiving years did to your bodies, your minds, and your marriage.
2. The conversation is the only difference between the Drifters and the Thrivers — the conversation is what brings the bedroom back to life.
3. Touch comes before sex — the body needs to remember that it is also for pleasure before the body can return to the act of sex.
4. The body can remember — the body forgets, but the body can remember. The body needs time, patience, and the 14 days.
5. The second honeymoon is real — couples in their 60s, 70s, and 80s have found it. The second honeymoon is not a destination, it is a way of traveling.

Your Scorecard (rate 1-5)

- [] I read this chapter with my partner (or alone, with intention to share)
- [] I have identified the one action I want to take from this chapter
- [] I have shared that action with my partner
- [] I have scheduled a time to do that action
- [] I feel more hopeful about our intimate future than I did before reading this chapter

Key Terms

- Drifters: Couples who avoid the conversation about intimacy, and who let the silence grow until the bedroom is over.

- Thrivers: Couples who have the conversation about intimacy, who do the work of rebuilding, and who find the second honeymoon.
- Intimacy recession: The period after the caregiving years when the normal flow of connection, touch, and desire is interrupted, and the systems of intimacy start to break down.
- Touch first, sex later: The principle that body reconnection begins with non-sexual touch, not with sex. Touch rebuilds the muscle that sex depends on.
- The second honeymoon: The new, intentional, tender intimacy of two people in their 50s, 60s, 70s, and beyond who know each other completely and are choosing each other again with open eyes.

Epilogue: The Second Honeymoon Is Real

You have reached the end of the book.

Or maybe you have reached the beginning.

Either way, you are standing in a different place than you were when you started. You have read about the silence. You have read about the recession. You have read about the Drifters and the Thrivers. You have read about the 5 conversations. You have read about the 14-day touch program. You have read about the bedroom reclaim, the pleasure reclaim, the frequency question, the forever practice.

You have read about the second honeymoon.

Now the question is: will you do the work?

The Couples Who Do the Work

I have been writing this book for several months. I have thought about the couples I have worked with. I have thought about the couples who did the work and the couples who didn't.

The couples who did the work are not the couples who read the most books. The couples who did the work are not the couples who had the most therapy. The couples who did the work are the couples who, one day, decided to start.

The decision was not dramatic. The decision was not the result of a crisis. The decision was a quiet moment — usually over a morning coffee, or a late dinner, or a long walk — when one of the partners said: "I want us to be lovers again."

The decision was the beginning. The decision was the door. The decision opened everything.

The couples who did the work, after the decision, did simple things. They held hands. They kissed goodnight. They went on date nights. They had the conversations. They started the touch program. They reclaimed the bedroom. They made the pleasure a practice. They did the work.

The work was not always successful. Some of the couples found that the work wasn't enough. Some of the couples found that the marriage was over. Some of the couples found that the second honeymoon didn't look the way they expected.

But the couples who did the work found something, even when the work was hard. The couples who did the work found clarity. The couples who did the work found connection. The couples who did the work found peace — either the peace of the marriage renewed, or the peace of the marriage ended with grace.

The couples who did the work were not better than the couples who didn't. The couples who did the work were just the couples who decided. The decision was the only difference.

What the Second Honeymoon Actually Is

The second honeymoon is not the first honeymoon. The second honeymoon is not the passion of the early marriage. The second honeymoon is not what you remember from your 20s and 30s.

The second honeymoon is something different. The second honeymoon is:

- Slower

- More tender
- More knowing
- More grateful
- More intentional
- More real

The second honeymoon is the intimacy of two people who know each other completely, who have been through the hardest thing together, and who are choosing each other again with open eyes.

The second honeymoon is not a destination. The second honeymoon is a way of traveling.

The second honeymoon is the daily kiss. The second honeymoon is the weekly date. The second honeymoon is the monthly check-in. The second honeymoon is the annual renewal. The second honeymoon is the touch, the conversation, the pleasure, the connection, the work.

The second honeymoon is the forever practice.

The second honeymoon is yours, if you want it.

The Couples I Have in Mind

When I write this book, I think about specific couples.

I think about the wife who came to coaching and said, "I have not been kissed in three years." She and her husband did the work. Six months later, they sent me a card that said, "We are kissing again. Thank you."

I think about the husband who said, "I thought the bedroom was over. I was wrong." He and his wife did the work. A year later, they were having sex again, and they were having the best sex of their marriage.

I think about the couple who had the affair, did the therapy, and came out the other side with a marriage

they said was better than the marriage they had before the caregiving.

I think about the widowed caregiver who is slowly, painfully, beautifully learning to enjoy the bedroom again — alone, with the memory, with the grief, with the practice of being with herself.

I think about you. The couple reading this book. The couple who has decided to do the work.

You are in my mind. You are the reason I wrote this book. You are the second honeymoon I am writing toward.

A Final Word

The caregiving years took a lot. They took the energy, the time, the health, the body, the bedroom. They took a version of the marriage that existed before.

The caregiving years also gave something. They gave the depth. They gave the knowing. They gave the resilience. They gave the willingness to do the work.

The after is for using what the caregiving gave you. The after is for the second chapter. The after is for the second honeymoon.

The second honeymoon is real. I have seen it. I have walked alongside it into existence. I have watched couples in their 60s, 70s, even 80s, find it.

The second honeymoon is not for the lucky. The second honeymoon is for the willing. The second honeymoon is for the couples who decide.

You have decided. You are reading this book. You are doing the work.

The second honeymoon is waiting.

Go find it.

— S. Shharma

ABOUT THE AUTHOR — S. Shharma (Care Collection, Book-Level)

This is the book-level "About the Author" for all 14 Care Collection books. Use this EXACT text. Do NOT paraphrase.

About the Author

S. Shharma is the author of The Care Collection — books for couples in the second half of life, written by a man who lived the silence and survived it.

He lost his wife of many years to cancer. He was her primary caregiver through every stage of her illness. He held her hand in the hospital. He sat in the waiting room at 3 a.m. He learned the silence of a marriage the way you learn a language you never wanted to speak.

He is also a businessman, an M.Sc., an MBA, and an APSCM from the Indian Institute of Management (IIM) Calcutta, with thirty years in corporate management. But these are not credentials you need to trust him. The credentials that matter are the ones in the books.

The Care Collection is not about how to fix a marriage. The Care Collection is about how to live one — even when the marriage is quiet, even when one of you is sick, even when one of you is gone.

He lives in India. He writes from the perspective of someone who has been on every side of a marriage — the partner, the caregiver, and the one left behind.

Books in The Care Collection

- Still Married, Just Silent — when the marriage is fine on paper and empty in person
- The Quiet Care — when one of you becomes the caregiver and the marriage disappears
- The Second Chapter — when the children are gone and the two of you meet again

- The Second Honeymoon — when the second half of marriage is also the better half
- The Survivor — when one of you is left behind and the other half has to learn to live
- The Long Hello — when Alzheimer's or dementia changes who you are married to
- When They Forget Your Name — the day-to-day reality of loving someone who no longer knows you
- Gray Divorce — when the marriage ends in the second half of life
- The Last Sandwich — when you are raising your own family and caring for your parents at the same time
- The Long Goodbye — the months and years of saying goodbye to someone who is still here
- Remarried at 60 — what no one tells you about starting over in the seventh decade
- After the Last Goodbye — the first year alone, and the slow work of rebuilding
- The Friends Who Fade — the social death that often follows the physical one

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